

PROPOSAL FORM – ROUND 9 (SINGLE COUNTRY APPLICANTS)

Applicant Name	The Tanzania National Coordinating Mechanism (TNCM)		
Country	The United Republic of TANZANIA - Mainland		
Income Level <i>(Refer to list of income levels by economy in Annex 1 to the Round 9 Guidelines)</i>	Low income country		
Applicant Type	☒ CCM	☐ Sub-CCM	☐ Non-CCM

Round 9 Proposal Element(s):			
Disease	Title	Does this disease include cross-cutting Health Systems Strengthening interventions in part 4B? <i>(include in <u>one</u> disease only)</i>	Is this a 're-submit' of the same disease proposal not recommended in Round 8?
HIV ¹	Enhancing HIV Prevention services in Tanzania	YES	NO
Tuberculosis ¹	Scaling up of Stop TB strategy		NO
Malaria	Sustaining malaria in Tanzania		NO

If this is a Round 8 proposal being re-submitted, have the TRP Review Form comments been clearly addressed in s.4.5.2?	☒ Yes	☐ No
Are there major new objectives compared to the Round 8 proposal that is being re-submitted? If yes, please provide a summary of the changes in the box below <u>by each disease re-submission and section number.</u>	☐ Yes	☐ No

¹ Different HIV and tuberculosis activities are recommended for different epidemiological situations. **For further information:** see the 'WHO Interim policy on collaborative TB/HIV activities' available at: http://www.who.int/tb/publications/tbhiv_interim_policy/en/

INSERT TEXT – maximum one page.

Currency	 USD	or	 EURO
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Deadline for submission of proposals:

**12 noon, Local Geneva Time,
Monday 1 June 2009**

INDEX OF SECTIONS and KEY ATTACHMENTS FOR PROPOSALS

'+' = A key attachment to the proposal. These documents **must** be submitted with the completed Proposal Form. Other documents may also be attached by an applicant to support their program strategy (*or strategies if more than one disease is applied for*) and funding requests. Applicants identify these in the 'Checklists' **at the end of** s.2 and s.5.

1. **Funding Summary and Contact Details**
2. **Applicant Summary (including eligibility)**
- + **Attachment C: Membership details of CCMs or Sub-CCMs**

Complete the following sections for each disease included in Round 9:

3. **Proposal Summary**
4. **Program Description**
4B. HSS cross-cutting interventions strategy **
5. **Funding Request**
5B. HSS cross-cutting funding details **

**** Only to be included in one disease in Round 9. Refer to the [Round 9 Guidelines](#) for detailed information.**

+ **Attachment A: 'Performance Framework'** (Indicators and targets)

+ **Attachment B: 'Preliminary List of Pharmaceutical and Health Products'**

+ **Detailed Work Plan:** Quarterly for years 1 - 2, and annual details for years 3, 4 and 5

+ **Detailed Budget:** Quarterly for years 1 - 2, and annual details for years 3, 4 and 5

IMPORTANT NOTE:

Applicants are strongly encouraged to read the [Round 9 Guidelines](#) fully before completing a Round 9 proposal. Applicants should continually refer to these Guidelines as they answer each section in the proposal form. All other Round 9 Documents are available [here](#).

A number of recent Global Fund Board decisions have been reflected in the Proposal Form. The [Round 9 Guidelines](#) explain these decisions in the order they apply to this Proposal Form. Information on these decisions is available at:

http://www.theglobalfund.org/documents/board/16/GF-BM16-Decisions_en.pdf.

Since Round 7, efforts have been made to simplify the structure and remove duplication in the Proposal Form. The [Round 9 Guidelines](#) therefore contain the **majority of instructions** and examples that will assist in the completion of the form.

1. FUNDING SUMMARY AND CONTACT DETAILS

Clarified Section 1.1

1.1. Funding summary

Disease	Total funds requested over proposal term					
	Year 1	Year 2	Year 3	Year 4	Year 5	Total
HIV	66,276,540	75,426,629	50,405,442	52,304,334	54,651,929	299064873
Tuberculosis	23,095,100	20,950,085	19,512,614	18,493,361	17,375,642	99,426,802
Malaria	36124729	39925794	40491420	29676466	27394200	173612609
HSS cross-cutting interventions section 4B and 5B within <i>[HIV and AIDS]</i>	42,209,087	55,692,858	30,263,305	23,632,223	24,292,505	176,089,978
Total Round 9 Funding Request ➔:						748,194,262

1.2. Contact details

	Primary contact	Secondary contact
Name	Dr. Bwijo A. Bwijo	Dr. Fatma Mrisho
Title	Secretary to the TNCM/Global Fund Coordinator	Executive Chairman
Organization	Tanzania Commission For AIDS (TACAIDS)	Tanzania Commission for AIDS (TACAIDS)
Mailing address	Tanzania Commission For AIDS P.O Box 76987, Dar es Salaam Tanzania	Tanzania Commission For AIDS P.O Box 76987, Dar es Salaam Tanzania
Telephone	+255 22 212 2651 Mobile: +255 784 530 325	+255 22 212 2651 Mobile: +255 784 530 325
Fax	+255 22 212 2427	+255 22 212 2427
E-mail address	bwijob@tacaids.go.tz	fmrisho@tacaids.go.tz
Alternate e-mail address	bwijob@yahoo.com.au	fmrisho@gmail.com

1.3. List of Abbreviations and Acronyms used by the Applicant

Acronym/ Abbreviation	Meaning
ABC	Abstinence, Be faithful, Condom use
ACO	Assistance Clinical Officer
ACSM	Advocacy, Communication and Social Mobilisation
ACT	Artemisinin Combination Therapy
ADDO	Accredited Drug Dispensing Outlet
ADR	Adverse Drug Reaction
AFB	Acid Fast Bacilli
AIDS	Acquired Immunodeficiency Syndrome
AMFm	Affordable Medicine Facility malaria
AMREF	African Medical Research Foundation
ANC	Ante-Natal Clinic
APHFTA	Association of Private Health Facilities of Tanzania
ART	Anti-retroviral Therapy
ARV	Anti-retroviral Therapy
ARD	Anti-retroviral Drug
ASAP	AIDS Strategy and Action Plan
BCC	Behavioural Change Communication
BMC	Bugando Medical Centre
BRELA	Business Registration and Licensing Authority
CBO	Community Based Organization
CCHP	Comprehensive Council Health Plan
CDC	Center for Disease Control
CEM	Cohort Event Monitoring
CHACS	Council HIV AIDS coordinator
CHMT	Council Health Management Team
CHSB	Council Health Services Board
CIDA	Canadian International Development Agency
CMO	Chief Medical Officer
CMT	Council Management Team
CO	Clinical Officer
CPT	Co-trimoxazole Prophylaxis Therapy
CSOs	Civil Society Organization
CSSC	Christian Social Services Commission
CTBC	Community Tuberculosis Care
CTC	Care and Treatment Centre
CTRL	Central Tuberculosis Reference Laboratory
CU	Central Unit
DACC	District AIDS Control Coordinator
DHIS	District Health Information System
DHS	Demographic Household Survey
DLDB	<i>Duka la Dawa Baridi</i> (unaccredited medicine kiosk)
DMO	District Medical Officer
DOTS	Directly Observed Treatment Short course
DP	Development Partner
DPS	Director of Preventive Services
DRS	Drug Resistance Survey

DST	Drug Susceptibility Testing
DSS	Demographic Surveillance Sites
DTLC	District Tuberculosis and Leprosy Coordinator
EHP	Emergency Hiring Programme
EQA	External Quality Assurance
EPI	Expanded Programme in Immunization
ETR	Electronic Tuberculosis Register
EU	European Union
FACs	Count machine for CD4 determination
FBO	Faith Based Organization
FDC	Fixed Drug Combination
FELTP	Field Epidemiology and Laboratory Training Program
FIFO	First In First Out
FTHP	Fast Track Hiring Project
GDF	Global Drug Facility
GFATM	Global Fund to Fight AIDS, Tuberculosis & Malaria
GFR 3	Global Fund Round 3
GFR 6	Global Fund Round 6
GF-Rd 9	Global fund Round 9
GFR 9	Global Fund Round 9
GLC	Green Light Committee
GLRA	German Leprosy and Tuberculosis Relief Agency
GoT	Government of Tanzania
GPHF	Germany Pharma Health Fund
HBS	Household Budget Survey
HC	Health Centre
HCW	Health Care Worker
HF	Health Facility
HE	Health Education
HIPIC	Highly indebted Poor Countries
HIS	Health Information System
HIV	Human Immunodeficiency Virus
HIV/SRH	HIV Sexual Reproductive Health
HMIS	Health Management Information System
HRD	Human Resources Development
HRH	Human Resource for Health
HRHSP	Human Resources for Health Strategic Plan
HSHP II	Health Sector HIV/AIDS Strategic Plan II
HSR	Health Sector Reforms
HSS	Health Systems Strengthening
HSSP	Health Sector Strategic Plan
ICB	International Competitive Bidding
ICT	Information Communication Technology
IDU	Intravenous Drug Users
IEC	Information Education and Communication
IFC	International Finance Corporation

IGA	Income Generating Activities
ILS	Integrated Logistics System
IMAI	Integrated Management of Adult Illnesses
IMCI	Integrated Management of Childhood Illness
IPTp	Intermittent Preventive Therapy in pregnancy
IPTp2	Intermittent Preventive Therapy in pregnancy second dose
IRS	Indoor Residual Spraying
ISTC	International Standard of Tuberculosis Care
IT	Information Technology
ITN	Insecticide Treated Net
IUATLD	International Union Against Tuberculosis and Lung Diseases
JICA	Japan International Cooperation Agency
KAP	Knowledge Attitude Practice
KCMC	Kilimanjaro Christian Medical Centre
KNCV	Royal Netherlands Tuberculosis Foundation
LED	Light Emitting Diode
LCD	Power Point Projector
LGA	Local Government Authority
LGDG	Local Government Development Grant
LLIN	Long-Lasting Insecticide Treated Net
LSR	Lead Sub Recipient
M&E	Monitoring and Evaluation
MASC	Medicine Access Steering Committee
MCH	Maternal and Child Health
MDA	Ministries, Departments, Agencies
MDG	Millennium Development Goals
MDR	Multi Drug Resistance
MFP	Malaria Focal Person
MNH	Muhimbili National Hospital
MOFEA	Ministry of Finance and Economic Affairs
MoHSW	Ministry of Health and Social Welfare
MOP	Malaria Operational Plan
MOU	Memorandum of Understanding
MP	Member of Parliament
MSD	Medical Stores Department
MSH	Management Sciences for Health
MTB	Ministerial Tender Board
MTEF	Medium Term Expenditure Framework
MTMSP	Medium Term Malaria Strategic Plan
MUKIKUTE	Ex TB Patients' Group in Temeke
MVU	Mobile Video Unit
NACP	National AIDS Control Programme
NATNETS	National Nets Programme
NBTS	National Multisectoral Strategic Framework
NCB	National Competitive Bidding
NFSD	Novartis Foundation for Sustainable Development
NGO	Non-Governmental Organization
NIMR	National Institute for Medical research
NMCP	National Malaria Control Program
NSGPR	National Strategy for Growth and Poverty Reduction
NTLP	National Tuberculosis and Leprosy Programme
OI	Opportunistic Infection
OPRAS	Open Performance Review and Appraisal System
OVC	Orphan and Vulnerable Children

OVC/MVC	Orphan and Vulnerable Children/Most Vulnerable Children
PAL	Practical Approach to Lung Health
PATH	Programme for Appropriate Technology in Health
PEPFAR	President's Emergency Plan for AIDS Relief
PER	Public Expenditure Review
PHC	Primary Health Care
PHSDP	Primary Health Services Development Programme
PITC	Provider Initiated Testing and Counselling
PLHIV	People Living with HIV
PMORALG	Prime Minister's Office Regional Administration and Local Government
PMS	Post Marketing Surveillance
PMSU	Procurement Management and Supplies Unit
PMU	Programme Management Unit
PMTCT	Prevention of Mother to Child Transmission
PR	Principle Recipient
PoE	Point of Entry
PHN	Public Health Nurse
PMCT	Prevention of Mother to Child Transmission
PMI	Presidents' Malaria Initiative
PMORALG	Prime Minister's Office Regional Administration and Local Government
POA	Programme of Work
PO-PSM	President's Office Public Service Management
PPA	Public Procurement Act
PPB	Patients per Bed
PS	Permanent Secretary
PSU	Pharmaceutical Services Unit
RACC	Regional AIDS Control Coordinator
RCC	Rolling Continuation Channel
R&R	Request and Report
RBM	Roll Back Malaria
RBTC	Regional Blood Transfusion Collection Sites
RCC	Rolling Continuation Channel
RDT	Rapid Diagnostic Test
RHMT	Regional Health Management Team
RMO	Regional Medical Officer
RTL	Regional Tuberculosis and Leprosy Coordinator
SAM	Service Availability Mapping
SDA	Service Delivery Area
SDC	Swiss Development Cooperation
SMS	Short Messaging Service (Text message)
SOP	Standard Operating Procedure
SR	Sub-recipient
SRH	Sexual Reproductive Health
STG	Standard Treatment Guidelines
STI	Sexually Transmitted Infection
TA	Technical Assistance
TACAIDS	Tanzania Commission for HIV/AIDS
TAF	Technical Assistance Fund
TANAM	Trustees of Tanzania National Malaria Movement
TB	Tuberculosis
TBA	Traditional Birth Attendant

TEHIP	Tanzania Essential Health Intervention Project
TFDA	Tanzania Food and Drugs Authority
TFNC	Tanzania Food and Nutrition Centre
THMIS	Tanzania HIV & Malaria Indicator Survey
TMARC	Tanzania Marketing and Communication
TNCM	Tanzania National Coordinating Mechanism
TNVS	Tanzania National Voucher Scheme
TOMSHA	Tanzania Output Monitoring System for HIV and AIDS
TOT	Trainer Of Trainer
TRP	Technical Review Panel
TSH	Thyroid Stimulating Hormone
TSH	Tanzanian Shillings
TSPA	Tanzania Service Provision Assessment
TTI	Transfusion Transmissible Infections
U5CC	Under 5 Catch-up Campaign
USG	Government of the United States
WHO	World Health Organization
ZTC	Zonal Training Center
UCSF	University of California San Francisco
UNICEF	United Nations Children's Fund
USAID	United States of America International Development
VCT	Voluntary Counseling and Testing
VNRBD	Voluntary Non Remunerable Blood donors
WHA	World Health Assembly
WHO	World Health Organisation
ZBTCs	Zonal Blood Transfusion Centers
ZHRC	Zonal Health Resource Centre
ZTC	Zonal Training Centre

2. APPLICANT SUMMARY (including eligibility)

CCM applicants: Only complete section 2.1. and 2.2. and [DELETE](#) sections 2.3. and 2.4.
Sub-CCM applicants: Complete sections 2.1. and 2.2. and 2.3. and [DELETE](#) section 2.4.
Non-CCM applicants: Only complete section 2.4. and [DELETE](#) sections 2.1. and 2.2. and 2.3.

IMPORTANT NOTE:

Different from Round 7, 'income level' eligibility is set out in s.4.5.1 (focus on poor and key affected populations depending on income level), and in s.5.1. (cost sharing).

2.1. Members and operations

2.1.1. Membership summary

Sector Representation	Number of members
☐ Academic/educational sector	1
☐ Government	5
☐ Non-government organizations (NGOs)/community-based organizations	2
☐ People living with the diseases	2
☐ People representing key affected populations ²	0
☐ Private sector	1
☐ Faith-based organizations	2
☐ Multilateral and bilateral development partners in country	3
☐ Other <i>(please specify)</i> : • Trade Union Congress of Tanzania (TUCTA) • Media Council of Tanzania	2
Total Number of Members: <i>(Number must equal number of members in 'Attachment C'³)</i>	18

² Please use the [Round 9 Guidelines](#) definition of *key affected populations*.

³ **Attachment C** is where the CCM (or Sub-CCM) lists the names and other details of all current members. This document is a mandatory attachment to an applicant's proposal. It is available at: http://www.theglobalfund.org/documents/rounds/9/CP_Pol_R9_AttachmentC_en.xls

2.1.2. Broad and inclusive membership

Since the last time you applied to the Global Fund (and were determined compliant with the minimum requirements):		
(a) Have non-government sector members (<i>including any new members since the last application</i>) continued to be transparently selected <u>by their own sector</u> ; and	☐ No	☒ Yes
(b) Is there continuing active membership of people living with and/or affected by the diseases.	☐ No	☒ Yes

2.1.3. Member knowledge and experience in cross-cutting issues

Health Systems Strengthening The Global Fund recognizes that weaknesses in the health system can constrain efforts to respond to the three diseases. We therefore encourage members to involve people (from both the government and non-government) who have a focus on the health system in the work of the CCM or Sub-CCM.
(a) Describe the capacity and experience of the CCM (or Sub-CCM) to consider how health system issues impact programs and outcomes for the three diseases.
The Tanzania National Coordinating Mechanism (TNCM) has a multi-sectoral composition comprising of 18 members drawn from Government Ministries/Departments, Civil society [Non Governmental Organizations (NGOs), Faith-Based Organizations (FBOs), People Living with HIV/AIDS (PLHIV), Academic Institutions], Private Sector Organizations [AIDS Business Coalition Tanzania (ABCT), Trade Union Congress Tanzania (TUCTA), Media Council of Tanzania and Development Partners. The TNCM composition has not changed from last year 2008 and has continued to build her capacity. These form the Executive Committee of TNCM. A significant proportion of these members have a health professional background with some of these having service delivery, and some have a long experience working with health related issues. Overall these members have awareness of the health sector system (HSS) of the country and are familiar with the various constraints therein and how these challenges can impact implementation of activities related to HIV prevention and control. In addition to the Executive Committee members, TNCM also has a technical group of experts who include Programme Managers of the three diseases targeted for funding in Global Fund (i.e. HIV/AIDS, TB and Malaria) and representatives from implementing partners as well as other constituencies. Their day to day activities place these technical experts at the core of the health system and as such they are conversant with all key health system issues and are very much aware of the various impediments in the system that interfere with smooth service delivery.

Gender awareness

The Global Fund recognizes that inequality between males and females, and the situation of sexual minorities are important drivers of epidemics, and that experience in programming requires knowledge and skills in:

- methodologies to assess gender differentials in disease burdens and their consequences (including differences between men and women, boys and girls), and in access to and the utilization of prevention, treatment, care and support programs; and
- the factors that make women and girls and sexual minorities vulnerable.

- (b) Describe the capacity and experience of the CCM (or Sub-CCM) in gender issues including the number of members with requisite knowledge and skills.

Current members ratio: Consideration of gender has been taken into account into the composition of the current TNCM, where there are 6 (33.3%) women and 12 (66.7%) men.

Plans on membership equal representation: A 50:50 representation is the ideal that Tanzania aims at. Unfortunately, at present predominance of males still continues due to the fact that even in the sectors where members are drawn from, women are fewer than men. This limitation notwithstanding, constituencies have been asked to encourage women to apply during renewal of TNCM membership. Selected members usually have basic gender skills with regard to existing gender inequalities between male and female. Even with the fewer number of women there is quality representation.

Knowledge and experience level: The need to be conversant with gender issues and address them in the day to day activities of sectors and institutions is well established in Tanzania. Apart from the global and national gender movement, all key development framework documents such as the United Nations Millennium Development Goals, Tanzania Development Vision 2025, Tanzania National Strategy for Growth and Poverty Reduction consider gender dimension as an important issue to be addressed comprehensively. Other documents include Gender Audit on Tanzania National Response on HIV and AIDS which was commissioned (February 2009) by the TACAIDS which serves as TNCM secretariat. All TNCM members are familiar with these documents as well as the several initiatives which have been undertaken to address gender concerns generally and in the context of specific health issues. In the operations of TNCM, provisions have been made to orient members on gender issues at the beginning of their tenure and during proposal development. Members also acquire vast experience in addressing gender issues in the various GF proposals and concepts that are processed by the experts.

Multi-sectoral planning

The Global Fund recognizes that multi-sectoral planning is important to expanding country capacity to respond to the three diseases.

- (c) Describe the capacity and experience of the CCM (or Sub-CCM) in multi-sectoral program design.

Since 2001, the United Republic of Tanzania moved from the health sector approach in responding to HIV/AIDS and institutionalized the multi-sectoral approach which is coordinated by the Tanzania Commission for AIDS (TACAIDS). Through this approach, the principle of the Three Ones (i.e. one coordinating body, one framework and one monitoring system) provides a guideline and base for all HIV/AIDS programme implementers.

Both the Executive Committee and the Technical Working Group members of TNCM are drawn from various sectors where they serve in senior positions. They are therefore experienced in across sector interaction and have over the years accumulated experience in developing, implementing and coordinating programs that involve multi-sectoral participation.

2.2. Eligibility

2.2.1. Application history

<i>'Check' one box in the table below and then follow the further instructions for that box in the right hand column.</i>	
☒ Applied for funding in Round 7 and/or Round 8 and was determined as having met the minimum eligibility requirements.	→ Complete all of sections 2.2.2 to 2.2.8 below.
☐ Last time <u>applied</u> for funding was before Round 7 or was determined non-compliant with the minimum eligibility requirements when last applied.	→ First, go to 'Attachment D' and complete. → Then also complete sections 2.2.5 to 2.2.8 below (Do not complete sections 2.2.2 to 2.2.4)

2.2.2. Transparent proposal development processes

- Refer to the document ['Clarifications on CCM Minimum Requirements'](#) when completing these questions.
- Documents supporting the information provided below must be submitted with the proposal as clearly named and numbered annexes. Refer to the 'Checklist' after s.2.

(a) Describe the process(es) used to invite submissions for possible integration into the proposal from a broad range of stakeholders <u>including civil society and the private sector, and at the national, sub-national and community levels.</u> <i>(If a different process was used for each disease, explain each process.)</i>
Tanzania National Coordinating Mechanism (TNCM) used three approaches to get ideas and interventions to be included in the Round 9 proposal. 1. A call through public and private media was made to get concept notes on interventions to be incorporated in Round 9. 2. Areas of focus was identified by TNCM and circulated to different stakeholders and constituencies to provide inputs, comments and include other interventions. 3. A two days panel discussion was held following the two days orientation on the guidelines, where more than 34 participants from 15 governmental and nongovernmental organizations, to discuss on priority interventions.
(b) Describe the process(es) used to transparently review the submissions received for possible integration into this proposal. <i>(If a different process was used for each disease, explain each process.)</i>
Largely the proposal development was guided by the focus areas endorsed by TNCM and inputs of the various consultative meetings. Although an advertisement by media was made, there was no concept notes received from nongovernmental organizations.
(c) Describe the process(es) used to ensure the input of people and stakeholders <u>other than CCM (or Sub-CCM) members</u> in the proposal development process. <i>(If a different process was used for each disease, explain each process.)</i>
A guiding team drawn from different TNCM consistencies, particularly the civil society and private sectors was established to oversee the development of the proposal and ensure the incorporation of views and interests of multiple stakeholders. As part of the proposal writing team many organizations including non CCM members have participated which allowed them to provide their contribution. Furthermore, the draft proposal was circulated earlier to different partners, both CCM and non CCM and in which were incorporated in the final draft of the proposal. During the endorsement of the proposal by the TNCM, other partners also attended the discussion and the presentation made by the proposal writing team

which also provided the opportunity for different stakeholders to provide additional inputs.	
(d) Attach a signed and dated version of the minutes of the meeting(s) at which the members decided on the elements to be included in the proposal for all diseases applied for.	<i>[Insert Annex Number]</i>

2.2.3. Processes to oversee program implementation

(a) Describe the process(es) used by the CCM (or Sub-CCM) to oversee program implementation.
Three methods have been used to oversee the implementation of the GF programs and PRs performance. A. Getting quarterly report from the PRS and reviewing the performance and financial expenditure reports. The Executive Committee reviews the quarterly executive dashboards to monitor grant implementation and performance to identify areas that are performing well and the ones that requires greater effort and /corrective measures. Quarterly meetings also bring together key decision makers across Government (4 Permanent Secretaries), and a team of technical people from relevant stakeholders. Each program is required to present progress and address key issues. B. Field visit to implementers/sub recipients and regions. NCM has started recently to do field visit to observe implementation of activities on the ground and verify reports. C. Commissioning of study when necessary. In 2008 a study was done to look at Global Fund implementation bottlenecks including flow and utilization of funds. The findings were presented in TNCM meeting and recommendations endorsed for implementation in June 2008; implementation of the recommendations has been started.
(b) Describe the process(es) used to ensure the input of stakeholders <u>other than CCM (or Sub-CCM) members</u> in the ongoing oversight of program implementation.
The operational manual of the TNCM has the provision to include non CCM members in the oversight of the grant implementation. They participate as observers in the CCM meeting and provide comments on the reports comparing to the ground information they have; they also participate in the field visit along with the CCM and the technical working groups.

2.2.4. Processes to select Principal Recipients

The Global Fund recommends that applicants select both government and non-government sector Principal Recipients to manage program implementation. → Refer to the [Round 9 Guidelines](#) for further explanation of the principles. .

(a) Describe the process used to make a transparent and documented selection of each of the Principal Recipient(s) nominated in this proposal. <i>(If a different process was used for each disease, explain each process.)</i>
A call through media was made; 28 non governmental and private sectors applied for the PRs. TNCM has guidelines for screening of PRs which focuses on Programmatic and financial management skill, implementation experienced technical and monitoring and Evaluation. Based on this two had found to be capable to act as PR.TNCM discussed on the reviews and the recommendation and endorsed Family Health International and Tanzania Marketing and Communication company Ltd to be nongovernmental PRs. While the PR for the Public sector remained to be Ministry of Finance and Economic Affairs because of its legal mandate to receive external fund for the public sector and also on the record of its previous

performance and experience.	
(b) Attach the signed and dated minutes of the meeting(s) at which the members decided on the Principal Recipient(s) for each disease.	<i>[Insert Annex Number]</i>

2.2.5. Principal Recipient(s)

Name	Disease	Sector**
Ministry of Finance and Economic Affairs	HIV &AIDS, TB, Malaria and HSS	Government
Family Health International HIV &AIDS	HIV and AIDS	CSO
Tanzania Marketing and Communication	HIV and AIDS	CSO
<i>[use "Tab" key to add extra rows if needed]</i>		

**** Choose a 'sector' from the possible options that are included in this Proposal Form at s.2.1.1.**

2.2.6. Non-implementation of dual track financing

Provide an explanation below if at least one government sector <u>and</u> one non-government sector Principal Recipient have not been nominated for each disease in this proposal.
Not applicable

2.2.7. Managing conflicts of interest

(a) Are the Chair and/or Vice-Chair of the CCM (or Sub-CCM) from the same entity as <u>any</u> of the nominated Principal Recipient(s) for any of the diseases in this proposal?	☒ Yes <i>provide details below</i>
	☒ No <i>→go to s.2.2.8.</i>
(b) If yes, attach the plan for the management of actual and potential conflicts of interest.	☐ Yes <i>[Insert Annex Number]</i>

2.2.8. Proposal endorsement by members

Attachment C – Membership information and Signatures	Has 'Attachment C' been completed with the signatures of all members of the CCM (or Sub-CCM)?	☐ Yes
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2.3. Sub-CCM details

2.3.1. Status of Sub-CCM

Identify if the sub-national coordinating mechanism:

- (a) Operates under the authority of the CCM and focuses on a particular region or issue.

☐ *Answer s.2.3.2. and s.2.3.3.*

- (b) Claims an independent basis to operate without oversight of the CCM

☐ *Answer s.2.3.2. and s.2.3.4.*

2.3.2. Rationale

Why does a Sub-CCM approach represent an effective approach in the circumstances of your country?

ONE PAGE MAXIMUM

2.3.3. CCM Endorsement

- (a) **Attach** the signed and dated minutes of the **CCM meeting** at which the CCM agreed to endorse the Sub-CCM proposal.

[Insert Annex Number]

- (b) **Attach** a letter from the CCM Chair or Vice-Chair with the minutes.

[Insert Annex Number]

2.3.4. Justification of independence of Sub-CCM

Explain how the Sub-CCM has a right to operate without guidance from the CCM.

ONE PAGE MAXIMUM

2.4. Non-CCM Applicants [\[delete sections 2.1. to 2.3. and only complete s.2.4 below\]](#)

2.4.1. Sector of work

<i>(check one box only):</i>	
☐	Academic/educational sector
☐	Government
☐	Non-government organization (NGO)/community-based organizations
☐	People living with the diseases
☐	People representing key affected populations
☐	Private sector
☐	Faith-based organizations
☐	Other: <i>please specify:</i>

2.4.2. Status of Non-CCM applicant

(a) Identify the <u>main</u> justification for submitting a non-CCM proposal <i>(check one box only):</i>	
(i) Country in conflict, facing a national disaster or in a complex emergency situation	☐ Go to s.2.4.3.
(ii) Country that suppresses, or has not established partnerships, with civil society and non-governmental organizations	☐ Complete (b) below, and then s.2.4.3.
(iii) State without a national government, and not being administered by a recognized interim administration	☐ Go to s.2.4.3.
(b) If (ii) applies:	
- describe, in date order, all attempts to have activities from the non-CCM proposal included in the CCM's proposal, and the CCM's response; and - briefly explain why you will be able to do the work and achieve the outputs/outcomes when the CCM has not supported the proposal.	
TWO PAGE MAXIMUM	

2.4.3. Expected benefit of proposal

Briefly explain how the work included in this proposal (HIV, tuberculosis and/or malaria as relevant) addresses gaps in the existing country efforts.

ONE PAGE MAXIMUM

2.4.4. Non-CCM knowledge and experience in cross-cutting issues

Health Systems Strengthening

The Global Fund recognizes that weaknesses in the health system can constrain efforts to respond to the three diseases. We therefore encourage members to involve people (from both the government and non-government) who have a focus on the health system in the work of the applicant.

- (a) Describe the capacity and experience of the applicant to consider how health system issues impact programs and outcomes for the three diseases.

Gender awareness

The Global Fund recognizes that inequality between males and females, and the situation of sexual minorities are important drivers of epidemics, and that experience in programming requires knowledge and skills in:

- methodologies to assess gender differentials in disease burdens and their consequences (including differences between men and women, boys and girls), and in access to and the utilization of prevention, treatment, care and support programs; and
- the factors that make women and girls and sexual minorities vulnerable.

- (b) Describe the capacity and experience of the applicant in gender issues including the number of members with requisite knowledge and skills.

Multi-sectoral planning

The Global Fund recognizes that multi-sectoral planning is important to expanding country capacity to respond to the three diseases.

- (c) Describe the capacity and experience of the applicant in multi-sectoral program design.

2.4.5. Principal Recipient(s)

The Global Fund recommends that applicants select both government and non-government sector Principal Recipients to manage program implementation. → [Refer to the Round 9 Guidelines for further explanation of the principles.](#)

Name	Disease	Sector**

<i>[use "Tab" key to add extra rows if needed]</i>		
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**** Choose a 'sector' from the possible options that are included in this Proposal Form at s.2.1.1.**

2.4.6. Non-implementation of dual track financing

Provide an explanation below if at least one government sector and one non-government sector Principal Recipient have not been nominated for each disease in this proposal.

ONE PAGE MAXIMUM

2.4.7. Endorsement by Non-CCM Applicant

Position	Printed Full Name	Signature

Proposal checklist - Section 1 and 2

Section 2: Eligibility		List Annex Name and Number
CCM and Sub-CCM applicants		
2.2.2(a)	Comprehensive documentation on processes used to <u>invite</u> submissions for possible integration into the proposal (if different processes used for each disease, attach as separate annexes).	
2.2.2(b)	Comprehensive documentation on processes used to <u>review</u> submissions for possible integration into the proposal (if different processes used for each disease, attach as separate annexes).	
2.2.2(c)	Comprehensive documentation on processes used to ensure the input of a broad range of stakeholders in the proposal development process	
2.2.3(a)	Comprehensive documentation on processes to oversee grant implementation by the CCM (or Sub-CCM).	
2.2.3(b)	Comprehensive documentation on processes used to ensure the input of a broad range of stakeholders in grant oversight process.	
2.2.4(a)	Comprehensive documentation on processes used to select and nominate the Principal Recipient (such as the minutes of the CCM meeting at which the PR(s) was/were nominated). If different processes used for each disease, then explain.	
2.2.7	Documented procedures for the management of potential Conflicts of Interest between the Principal Recipient(s) and the Chair or Vice Chair of the Coordinating Mechanism	
2.2.8	Minutes of the meeting at which the proposal was developed and CCM (or Sub-CCM) endorsed.	
2.2.8	Endorsement of the proposal by all CCM (or Sub-CCM) members.	Attachment C to the Proposal Form
Sub-CCM applicants only		
2.3.3 (CCM Endorsement)	Documented evidence (including minutes of the CCM meetings) that the CCM in the country reviewed and endorsed the proposal (as relevant).	
2.3.4	Documented evidence justifying the Sub-CCM's right to operate without guidance from the CCM.	

Proposal checklist - Section 1 and 2

Non-CCM applicants only		
2.4.1	Documentation describing the organization such as statutes and by-laws (official registration papers) or other governance documents, documents evidencing the key governance arrangements of the organization, a summary of the organization, including background and history, scope of work, past and current activities, and a summary of the main sources and amounts of funding.	
2.4.2(a)	Documentary evidence justifying the one of the three exceptional circumstances for submitting a non-CCM proposal	
2.4.2(b)	Documentary evidence of any attempts to include the proposal in the relevant CCM's final approved country proposal and any response from the CCM.	
Other documents relevant to sections 1 and 2 attached by applicant: <i>(add extra rows to this section of the table as required to ensure that documents directly relevant are attached)</i>		

ROUND 9 – Tuberculosis

3 PROPOSAL SUMMARY

3.1 Duration of Proposal	Planned Start Date	To
Month and year: (up to 5 years)	July 2010	June 2015

3.2 Consolidation of grants	
(a) Does the CCM (or Sub-CCM) wish to consolidate any existing tuberculosis Global Fund grant(s) with the Round 9 tuberculosis proposal?	☐ Yes (go first to (b) below)
	☐ No (go to s.3.3. below)
'Consolidation' refers to the situation where multiple grants can be combined to form one grant. Under Global Fund policy, this is possible if the same Principal Recipient ('PR') is already managing at least one grant for the same disease. A proposal with more than one nominated PR may seek to consolidate part of the Round 9 proposal. → More detailed information on grant consolidation (including analysis of some of the benefits and areas to consider is available at: http://www.theglobalfund.org/documents/rounds/9/CP_Pol_R9_FAQ_GrantConsolidation_en.pdf	
(b) If yes, which grants are planned to be consolidated with the Round 9 proposal after Board approval? (List the relevant grant number(s))	

3.3 Alignment of planning and fiscal cycles

Describe how the start date:
(a) contributes to alignment with the national planning, budgeting and fiscal cycle; and/or
(b) in grant consolidation cases, increases alignment of planning, implementation and reporting efforts.
ONE PAGE MAXIMUM Alignment with National Plans The guiding framework for Tanzania's TB control response is the global plan of the Stop TB Strategy (2006 - 2015), which is supported by several other specific plans, including the NTLF strategic plan 2004 – 2009, the Health Sector HIV and AIDS Strategic Plan II 2008-2012 (HSHSP II), the Health Sector Strategic Plan III 2009 – 2013 (HSSP III), and the Human Resources for Health Strategic Plan 2008-2013 (relevant for the HRH component of HSS). Thus Tanzania has national-level plans that rally the Government, Development Partners (DPs) and other stakeholders towards a common globally accepted TB control framework. Global Fund proposals are developed to align with these national plans and priorities. In addition, efforts to mainstream gender are firmly on the agenda in all strategies and operational plans. This is in line with the National Gender Policy of 2000, the published report of the pilot study on TB and gender in Tanzania in 2004, as well as the National Gender Action Plan (2008). This proposal is fully aligned with these national strategies and builds on the work already undertaken as well as on support from non-GF sources. As such, this proposal is not a stand-alone initiative. It is anchored in the Government's Medium Term Expenditure Framework (MTEF), with the fiscal year running from July 1st to June 30th. It builds on previous GF support, fully integrated into other support from Development partners (DPs)—including the US Government's President's Emergency Plan for AIDS Relief (PEPFAR) program—and is complementary to national efforts led by the Tanzania Commission for HIV/AIDS (TACAIDS) and the National TB and Leprosy Programme (NTLP). TB control initiatives in Tanzania are in line with MoHSW's new Health Sector Strategic Plan (HSSP III), which goes beyond Tuberculosis. Extensive consultations were conducted during the development of this proposal to ensure that it corresponds with existing national plans and also helps to inform the policy process.

ROUND 9 – Tuberculosis

Alignment with the National Fiscal and Budgetary Cycles

Previous experience shows that it is not easy to align GF proposals with the national budgetary and fiscal cycles.

In all previous Global Fund grants there had been efforts to try to synchronize the grants with the national fiscal cycle right from planning to implementation, but historically this has remained a big challenge, even when the intent existed from proposal writing. As highlighted in the December 2007 Tanzania Public Expenditure Review (PER), these challenges have resulted in significant delays in disbursement of funds and their subsequent expenditure.

However, it would be highly appreciated if the timing of this proposal coincided with Tanzania's fiscal and budgetary cycle, since once approved, the grants are operationalized through medium-term expenditure framework (MTEF) plans, annual action plans and budgets for all Government ministries, departments, and agencies (MDAs).

As noted in 3.2 above, this proposal does not aim to consolidate with any of the previous TB grants, not only that there will be no other active TB grant by the time the grant becomes approved but also the involved technicalities. While the TNCM recognizes the value of integrating Global Fund resources into a common pool and process, the complexity of grant consolidation combined with the lack of experience among both Global Fund staff and Tanzania's Ministry of Finance and Economic Affairs (MoFEA) in grant consolidation influenced the decision to postpone grants consolidation even for those components that have active grants.

The TNCM plans to engage with the GFATM more frequently to address challenges of GF alignment and consolidations. The TNCM has started this process with discussions with the GF to synchronize the performance and financial reporting periods for GF grants with that of the fiscal cycle of the GoT.

3.4 Program-based approach for Tuberculosis

3.4.1. Does planning and funding for the country's response to tuberculosis occur through a program-based approach?	☐ Yes. Answer s.3.4.2
	☐ No. → Go to s.3.5.
3.4.2. If yes, does this proposal plan for some or all of the requested funding to be paid into a common-funding mechanism to support that approach?	☐ Yes → <i>Complete s.5.5 as an additional section to explain the financial operations of the common funding mechanism.</i>
	☐ No. Do not complete s.5.5

ROUND 9 – Tuberculosis

3.5 Summary of Round 9 Tuberculosis Proposal

Provide a summary of the tuberculosis proposal described in detail in section 4.

Prepare after completing s.4.

ONE PAGE MAXIMUM

Tuberculosis is among the most important infectious diseases in Tanzania with a high mortality, especially among the HIV infected. Tanzania is one of the large countries in Africa, covering nearly 1 million square kilometer and 40 million people, living in 10,342 villages, 2,555 wards, 133 district councils and 21 regions. Around 75% of the population lives in rural areas. Health service is provided by more than 5,000 health facilities (219 hospitals, 481 health centres and 4600 dispensaries; 65% are public facilities). Since 1977 the National Tuberculosis and Leprosy Programme (NTLP) guides TB control activities undertaken by the general health service, with specialist staff at national, regional and district level. The number of tuberculosis cases has increased from 11,753 in 1983 to 62,092 in 2007, a five-fold increase. Over the years TB control strategies and work plans have been adjusted to upcoming challenges and new insights. Tuberculosis control is in line with the Primary Health Care approach and national policies for poverty reduction. The Ministry of Health and Social Welfare has adopted the Stop TB Strategy, which is being implemented nation-wide with a penetration of 100% DOTS coverage.

The NTLP has been pursuing a five year (2004/2005 -2008/2009) strategic plan to provide high quality and effective interventions to control TB in Tanzania with a focus on gender mainstreaming, equity, accessibility and those most at risk, including the HIV infected. This plan is partly funded through the government's MTEF and various additional external financial sources: Tanzania has benefited from financial support of the Global Fund against AIDS, TB and Malaria in Rounds 3 and 6. These funds were used to strengthen TB and TB/HIV services in 50 districts in the country from 2004. Tanzania has also received funds from WHO, GLRA, PEPFAR through CDC Country office and PATH/USAID to scale up TB/HIV activities in the remaining 83 district councils. These resources have been used to strengthen TB and TB/HIV services in the country. Community involvement through treatment supporters was successfully introduced and expanded. During this period, the TB treatment success has increased from 81% in 2004 to 85% in 2007. Furthermore, collaborative TB/HIV activities, which started in 2005, have been scaled up throughout the country and by 2008 PITC reached 68%. During a Drug Resistance Survey in 2007 MDR-TB was found to be low (1.2% in new cases, 3% in re-treatment cases).

But results are sub-optimal due to weaknesses in the TB control programme and the health system at large. The following constraints and gaps were identified during the recent review of the NTLP 5 year strategic plan 2004-2009: **Quality DOTS coverage:** DOTS coverage 100% since 1986, but TB services in only 2/3 of all health facilities and not evenly distributed, causing lack of access to diagnosis through sputum smear or X-ray examinations and leading to suboptimal diagnosis of TB, especially in children. **Collaborative TB/HIV activities:** Shortfall in coverage of collaborative TB/HIV services, also in congregate settings, resulting in 72% of TB/HIV co-infected patients receiving CPT, but only 31% taking ART. Employment of TB/HIV officers by CDC /PEPFAR and PATH/USAID funded partners in more than half of the districts gave improvement, but coverage and impact is still insufficient. **Monitoring and Evaluation:** Inadequate recording and reporting of TB/HIV data observed at facility level; delay in transfer of data and reports from district to regional and national level. Limited analysis of data at district and regional level for decision making and feedback. **Supervision:** Supervision at national, regional, district and community level was weak, resulting in lack of oversight of TB control activities. **Anti-TB drugs management:** Shortcomings at various levels, routine procedures are not followed. **Laboratory network:** Gaps were identified in extent and quality of implementation of the lab external quality assurance (EQA). Lack of new and effective diagnostic technologies for diagnosis of TB. **MDR TB:** Routine surveillance system for MDR TB is weak; current estimated numbers of patients to be treated are unreliable. Capacity at the Kibong'oto facility considered insufficient. 50% of MDR-TB patients found in Dar es Salaam. **Advocacy Communication and Social Mobilization (ACSM):** Shortage of qualified ACSM staff to address the low community awareness of tuberculosis. **Operational research:** Lack of an operational research strategy and slow progress (and a financial gap) for the national TB prevalence survey. The **health system** as a whole has severe weaknesses in human resource, the approach to lung health, infrastructure, health information systems, procurement and supply management as well as leadership and governance.

ROUND 9 – Tuberculosis

As the mission of the NTLP is to provide high quality and effective interventions to control TB with a focus on gender mainstreaming, equity, accessibility and those most at risk, the main goal of the TB component proposal in Round 9 is to achieve universal access to quality TB care including strengthening of diagnostic services and patient-centred treatment.

The NTLP will focus on reducing the burden of tuberculosis by further improving access, increasing case-detection, addressing TB/HIV and MDR-TB in line with the Stop TB Strategy (2006 - 2015). The shortcomings in case finding will be addressed through the introduction of new diagnostic tools, intensified case finding and engaging more private providers and the community in TB service provision. The programme will also put emphasis during pre- and in-service training on TB in children and introduction of International Standard of Tuberculosis Care (ISTC) and Practical Approach to Lung health (PAL). Collaborative TB/HIV activities will be strengthened with special focus on congregate settings. Another priority area will be to strengthen MDR-TB management. The targets are 70% case detection by 2014, 98% of TB patients tested for HIV and scaling up MDR-TB treatment to reach 25% of the survey-based estimated number.

This component shall be implemented in a collaborative manner from national level to regions, districts and communities involving both public and private health facilities with technical leadership from the NTLP. The MOHSW shall involve the Ministry responsible for Local Government and District Councils under the Prime Minister's Office to ensure relevant activities are reflected in Comprehensive Council Health Plans. Complementary to this fund request continued funding is expected from the Government of Tanzania and other collaborating partners including CIDA/KNCV, NFSD, GLRA, CDC/PEPFAR and technical support from WHO.

Total estimated budget for the component is US\$ 99,426,802.

ROUND 9 – Tuberculosis

4 PROGRAM DESCRIPTION

4.1 National program and strategy

(a) Briefly summarize:

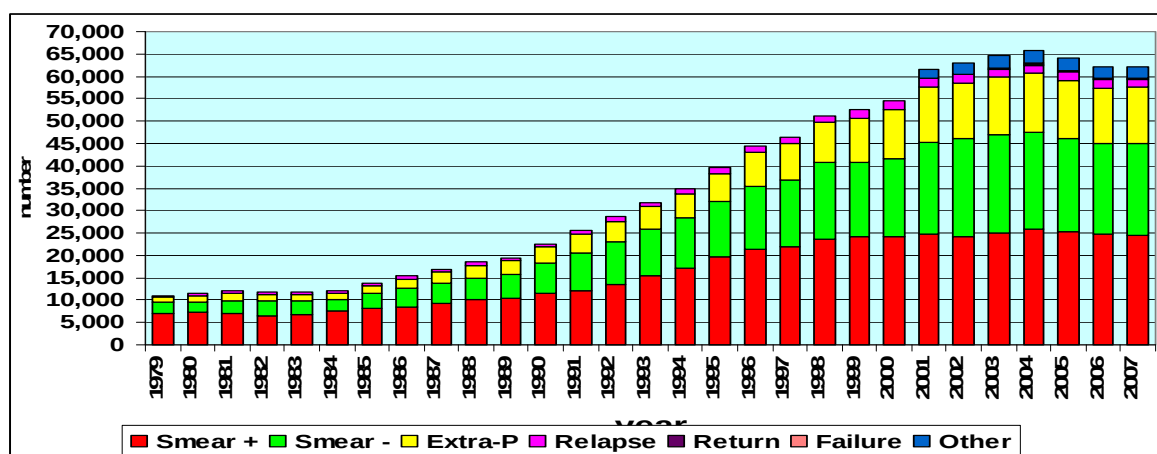
- the current tuberculosis national program or strategy;
- how the strategy responds comprehensively to current epidemiological situation in the country; and
- the improved tuberculosis outcomes expected from implementation of these programs or strategy.

ONE PAGE MAXIMUM

Tuberculosis is among the most important infectious diseases in Tanzania with a high mortality, especially among the HIV infected. The National Tuberculosis and Leprosy Programme exist over 30 years, with control activities undertaken by the general health service. The programme is guided by specialists from Central Unit (CU) at the MoHSW, with 11 technical (36 total) staff; by 26 Regional TB/Leprosy Coordinators and 158 District TB/L Coordinators (some administrative districts have more than one DTLC). In the course of years TB control strategies and work plans have been developed to adjust to upcoming challenges and new insights. Tuberculosis control in the country is in line with the National Strategy for Economic Growth and Poverty Reduction and also consistent with the new Stop TB Strategy which is implemented nation-wide with a penetration of 100% DOTS coverage.

Tuberculosis burden in Tanzania ranks third among major causes of morbidity and mortality after malaria and AIDS¹. The number of tuberculosis cases has increased from 11,753 in 1983 to 62,092 in 2007 which is over five-fold increase. The annual increase in case notification over the last two decades was 2%-5%, largely attributable to the HIV epidemic and population increase.

Figure: Trend of TB cases notification in Tanzania 1979 - 2007



The number of smear positive cases in the cohort notified in 2007 was 40% of the total TB cases, which constitutes 51% of the estimated smear positive pulmonary TB cases.

The National Tuberculosis and Leprosy Programme has been pursuing a five year (2004/2005 - 2008/2009) strategic plan to provide high quality and effective interventions to control TB in Tanzania with a focus on gender mainstreaming, equity, accessibility and those most at risk, including the HIV infected. This plan is partly funded through the government's MTEF, while Tanzania has benefited from various additional external financial sources. These resources have been used to strengthen TB and TB/HIV services in the country. During this period, the treatment success has increased from 81% in 2004 to 85% in 2007. Furthermore, collaborative TB/HIV activities which started in 2005 have been scaled up throughout the country and by 2007 50% of all TB cases were counselled and tested for HIV. In 2008 PITC improved further the uptake of HIV testing among TB patients to 68% (draft review report). MDR-TB was found to be low (1.2% in new cases, 3% in re-treatment cases) during a Drug Resistance Survey in 2007.

¹ Health Abstract 2006/AMMP final report 2004

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The NTLP will focus on reducing the burden of tuberculosis by further improving access, increasing case-detection, addressing MDR-TB and TB/HIV in line with the Millennium Development Goal 6, the Health Sector Strategic Plan III, the National Strategy for Growth and Reduction of Poverty (NSGPR) and the Stop TB Strategy (2006 - 2015). The shortcomings in case finding will be addressed through the introduction of new diagnostic tools, intensified case finding and engaging more private providers and the community in TB service provision. The programme will also put emphasis during pre- and in-service training on TB in children and introduction of International Standard of Tuberculosis Care (ISTC) and Practical Approach to Lung health (PAL). The target are 70% case detection by 2014, 98% of TB patients tested for HIV and scaling up MDR-TB treatment.

- (b) From the list below, attach *** only those documents that are directly relevant** to the focus of this proposal (or, **identify the specific Annex number from a Round 7 or Round 8 proposal when the document was last submitted, and the Global Fund will obtain this document from our files*).

Also identify the specific page(s) (in these documents) that support the descriptions in s.4.1. above.

Document	Proposal Annex Number	Page References
☒ National Health Sector Development/Strategic Plan	1	15, 20 and 25
☒ National Tuberculosis Control Mid Term Strategy or Plan	2	
☒ National Tuberculosis Guidelines (medical and laboratory)	3	
☒ Important sub-sector policies that are relevant to the proposal (e.g., national or sub-national human resources policy, or norms and standards)	4	
☒ Most recent annual reports, monitoring mission reports or reviews, including any epidemiology report directly relevant to the proposal (2009 review report and 2007 annual report)	5	
☐ National Monitoring and Evaluation Plan (health sector, disease specific or other)		
☐ National policies to achieve gender equality in regard to the provision of tuberculosis diagnosis, treatment, and care and support services to all people in need of services		

ROUND 9 – Tuberculosis

4.2 Epidemiological Background

4.2.1. Geographic reach of this proposal		
(a) Do the activities target:		
 Whole country	 Specific Region(s) <i>**If so, insert a map to show where</i>	 Specific population groups <i>**If so, insert a map to show where these groups are if they are in a specific area of the country</i>

**** Paste map here if relevant**

(b) Size of population group(s) (If national data is disaggregated differently then type over the categories proposed)			
Population Groups	Population Size	Source of Data	Year of Estimate
Total country population (all ages)	40,667,794	Tanzania National Projection (vol XII) National Bureau of statistics	2008
Women > 25 years	7,685,903	Tanzania National Projection (vol XII) National Bureau of statistics	2008
Women 20 – 24 years	2,230,736	Tanzania National Projection (vol XII) National Bureau of statistics	2008
Women 15 –19years	1,796,292	Tanzania National Projection (vol XII) National Bureau of statistics	2008
Men > 25 years	6,894,673	Tanzania National Projection (vol XII) National Bureau of statistics	2008
Men 20 – 24 years	2,193,777	Tanzania National Projection (vol XII) National Bureau of statistics	2008
Men 15 – 19 years	1,815,100	Tanzania National Projection (vol XII) National Bureau of statistics	2008
Girls 0 – 14 years	8,983,919	Tanzania National Projection (vol XII) National Bureau of statistics	2008
Boys 0 – 14 years	9,067,394	Tanzania National Projection (vol XII) National Bureau of statistics	2008
Girls < 5 years	3,686,776	Tanzania National Projection (vol XII) National Bureau of statistics	2008
Boys < 5 years	3,750,164	Tanzania National Projection (vol XII) National Bureau of statistics	2008
Girls 5 - 14 years	5,297,143	Tanzania National Projection (vol XII) National Bureau of statistics	2008

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(b) Size of population group(s) <i>(If national data is disaggregated differently then type over the categories proposed)</i>			
Population Groups	Population Size	Source of Data	Year of Estimate
Boys 5 - 14 years	5,317,230	Tanzania National Projection (vol XII) National Bureau of statistics	2008
Other **: <i>**Refer to the Round 9 Guidelines for other possible groups</i>		Check with other groups Otherwise website	
Other **:			
Other **:			<i>[use "Tab" key to add extra rows if needed]</i>

4.2.2. Tuberculosis epidemiology of target population(s)				
Indicators (see the footnote under this table for the references)			Number or rate or percentage	[Calculation] or (reference)
TB estimates, 2007				
a	Estimated number of new TB cases (all forms)		120,000	(1)
		Male 0-14 (5.4% of total number)	6,480	[a *5.4%]
		Female 0-14 (6.5% of total number)	7,800	[a*6.5%]
b	Estimated number of new TB cases (all forms) per 100 000 population		297	[a/population*100 000]
c	Estimated number of new smear-positive cases		49,000	(1)
d	Estimated number of new smear-positive cases per 100 000 population		120	[c/population*100 000]
e	Estimated prevalence of TB cases (all forms)		136,000	(1)
f	Estimated prevalence of TB cases (all forms) per 100 000 population		337	[e/population*100 000]
g	Estimated number of deaths due to TB (all forms)		32,000	(1)
h	Estimated number of deaths due to TB (all forms) per 100 000 population		78	[g/population*100 000]
i	Estimated number of HIV-positive new TB cases (all forms)		56,000	(1)
j	Estimated number of HIV-positive new TB cases (all forms) per 100 000 population		139	[i/population*100 000]
k	Estimated number of multi-drug resistant patients of TB (new and re-treatment cases combined)		1,335	(2)
ka	Estimated % of TB cases (new and re-treatment combined) that are multi-drug resistant		1.0	(2)
TB notifications, 2007				
l	Number of new TB cases (all forms) notified		57,567	(3)
		Male 0-14	2,780	(3)
		Male , 15 and more	30,656	(3)

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	Female 0-14	2,519	(3)
	Female, 15 and more	21,612	(3)
m	Number of new TB cases (all forms) notified per 100 000 population	147	[l/population*100 000]
n	% of estimated new TB cases (all forms) notified	48	[l/a*100]
o	Number of new smear-positive TB cases notified	24,520	(3)
	Male 0-14	189	(3)
	Male, 15-44	10,544	(3)
	Male, 45 and more	4,611	(3)
	Female 0-14	237	(3)
	Female 15-44	7,071	(3)
	Female, 45 and more	1,868	(3)
p	Number of new smear-positive TB cases notified per 100 000 population	62	[o/population*100 000]
q	% of estimated new smear-positive TB cases notified - Case detection rate of new smear positive TB	51	[o/c*100]
s	Number of TB cases all forms (new and re-treatment) that were tested for HIV	31,305	(3)
t	% of TB cases all forms (new and re-treatment) that were tested for HIV	50	[s/l*100]
u	Number of notified TB cases all forms (new and re-treatment cases) that were found or known to be HIV-positive	14,669	(3)
v	% of all estimated HIV-positive TB cases that were found or known to be HIV-positive - case detection of HIV+ TB	26	[u/i*100]
w	Number of notified HIV-positive TB cases (new and re-treatment) started or continued on CPT	10,541	(3)
x	% of all notified HIV-positive TB cases (new and re-treatment) started or continued on CPT	72	[w/u*100]
y	Number of notified HIV-positive TB cases new and re-treatment) started or continued on ART	4,619	(3)
z	% of all notified HIV-positive TB cases (new and re-treatment) started or continued on ART	32	[y/u*100]
aa	Number of TB cases (new and re-treatment) received diagnostic DST	169	(3)
ac	Number of multi-drug resistant TB (MDR-TB) cases notified among new and re-treatment cases	25	(3)
ad	% of all estimated MDR-TB cases that were found or known as MDR-TB - case detection MDR-TB	2	[ac/k*100]
Treatment outcome, 2006			
ae	Number of new smear-positive cases registered for treatment	24,724	(3)
af	% of all notified new smear-positive TB cases that were registered for treatment (2006 cohort)	100	[ae/o*100]
ag	Number of new smear-positive TB cases that were successfully treated (2006 cohort)	20,949	(3)
ah	% of all new smear-positive TB cases registered for treatment that were successfully treated (2006 cohort)	85	[ag/ae*100]
ai	Number of new smear positive TB cases that failed their	61	(3)

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	treatment		
aj	% of all new smear-positive TB cases registered for treatment who failed their treatment (2006 cohort)	0.2	[ai/ae*100]
ak	Number of new smear positive TB cases who died while on TB treatment	1,954	(3)
al	% of all new smear-positive TB cases registered for treatment who died while on TB treatment (2006 cohort)	8	[ak/ae*100]
am	Number of new smear positive TB cases who defaulted	780	(3)
an	% of all new smear-positive TB cases registered for treatment who defaulted (2006 cohort)	3	[am/ae*100]
Other			
Other			
Other			[use "Tab" key to add extra rows if needed]

1. Global tuberculosis control: surveillance, planning, financing: "WHO/HTM/TB/2008.393".
2. Anti-tuberculosis drug-resistant in the world, Fourth global report. WHO/HTM/TB/2008.394
3. Data from country TB routine recording and reporting system.

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4.3. Major constraints and gaps

(For the questions below, consider government, non-government and community level weaknesses and gaps, and also any key affected populations² who may have disproportionately low access to tuberculosis diagnosis, treatment, and care and support services, including women, girls, and sexual minorities.)

4.3.1. Tuberculosis program

Describe:

- the main weaknesses in the implementation of current tuberculosis program or strategy;
- how these weaknesses affect achievement of planned national tuberculosis outcomes; and
- existing gaps in the delivery of services to target populations.

In February 2009 the National Tuberculosis and Leprosy Programme (NTLP) conducted an end term evaluation of the third NTLP Medium Term Strategic Plan (2004 – 2009). The review assessed the implementation status of the third NTLP Strategic Plan. Weaknesses in the following areas were identified based on the preliminary review report, Health Sector Strategic Plan III (2009 – 2015) and routine NTLP implementation experience. The fourth NTLP Medium Term Strategic Plan (2009/2010 – 2014/2015) will be developed in the next few months, taking these weaknesses into account while defining the priorities in line with the Stop TB Strategy:

Quality DOTS coverage: Even though 100% geographical DOTS coverage was achieved in Tanzania since 1986, the Tanzania Service Availability Mapping (SAM, conducted in 2006) revealed that TB services are available in only about 66% of all health facilities and not evenly distributed throughout the country. Many of the rural health facilities are understaffed, hampering the (passive) case finding. The lack of access for part of the population in remote areas might contribute to the WHO estimated low detection rate and the inability to successfully treat all detected tuberculosis patients. Contacts of known TB patients are often not examined. Lack of access to X-ray examinations reduces detection of sputum smear negative and extra-pulmonary TB, leading to suboptimal diagnosis of TB in children as well.

Collaborative TB/HIV activities: There is a shortfall in awareness and coverage of collaborative TB/HIV services in the districts, including in congregate settings, demonstrated by the fact that just 50% of TB patients were counseled and tested for HIV in 2007. At the service provision level lack of qualified human resources, among other things, has resulted in 72% of TB/HIV co-infected patients receiving CPT but only 31% taking ART. The employment by PEPFAR funded partners (PATH, CDC) of TB/HIV officers in more than half of the districts may have alleviated the situation already, but coverage and impact is insufficient still. Furthermore, awareness and implementation of TB case finding in HIV/AIDS care settings is low, the TB screening tool is hardly used. Isoniazid Preventive Treatment is planned to be implemented in only a few districts. Infection control measures (administrative and infrastructure) have been started in few places under PEPFAR support, however, the extent is not known to the NTLP. In addition to this, coordination between the NTLP and NACP at national level has not yet been operationalized.

Monitoring and Evaluation: Inadequate recording and reporting of TB/HIV data is observed at facility level while NTLP data collection tools, including the electronic TB register, have been updated to conform largely to current WHO requirements. Unavailability of computers and accessories is noted in 34 districts, compounded by lack of basic computer knowledge and maintenance including antivirus programmes. This has significantly contributed to delay of transfer of data and reports from district to regional and national level. Analysis of data at district and regional level for decision making and feedback is very limited.

Supervision: Supervision at all levels (national, regional, districts and community) was identified as a weakness in the 2009 NTLP end term review. This has resulted in a lack of oversight of TB control activities, including inadequate laboratory diagnosis (insufficient quality assurance of AFB microscopy), TB/HIV integration and stock management of Anti-TB drugs. Programme supervision is affected by competing demands on the few available staff who are assigned multiple responsibilities. Since the inception of the NTLP in 1977, the German Leprosy and TB Relief Agency (GLRA) has been providing

² Please refer back to the definition in s.2 and found in the [Round 9 Guidelines](#).

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vehicles and motorcycles for TB and Leprosy supervision. Currently the support entails 30 4WD vehicles and 159 motorcycles bought at different times. Unfortunately, this traditional support from GLRA has stopped, as donations to GLRA have diminished. Interventions to ensure efficient human resources and maintenance of the programme's transport network in order to facilitate effective and sustainable supportive supervision at all levels will be of paramount importance for the programme.

Anti-TB drugs management: The recent NTLP review observed a number of shortcomings in the management of anti-TB drugs at various levels. Gaps were noted in recording of receipts, keeping stock ledgers and BIN cards up to date, use of issue vouchers; FIFO was not practised everywhere, resulting in some drugs expiring on the shelves. The management of anti-TB drugs is fully integrated into the general health system. These are health system issues, to be addressed more widely.

Laboratory network: Lack of new and effective diagnostic technologies for diagnosis of TB at all levels and shortage of skilled health workers are also contributors to low TB case detection rates (51% versus WHA target of 70% by 2005). There is widespread use of the light microscope, but this can detect only 50% of all TB suspects. Gaps were identified in extent and quality of implementation of the lab external quality assurance (EQA).

MDR TB: The surveillance system for MDR TB in the country is still weak, making the currently estimated numbers of patients to be treated unreliable. There will be inadequate capacity at the Kibong'oto facility in Kilimanjaro Region to ensure universal access to MDR TB management, when the WHO estimates would be correct. More than 50% of MDR-TB patients found during the drug resistance survey are from Dar es Salaam. Through this proposal, the plan is to establish a satellite MDR TB treatment facility in Dar es Salaam to accommodate the scale up of MDR TB services and refurbish TB/HIV clinics to conform to infection control measures, respect privacy and improve the working conditions for healthcare workers.

Advocacy Communication and Social Mobilization (ACSM): Two studies (PHC Iringa 2006 and Ifakara/Novartis 2007) showed a low community awareness of tuberculosis. NTLP experienced shortage of qualified staff to make use of GFR6 funding for ACSM, but has recently, in 2009, built capacity by employing ACSM staff at the CU under GFR6 support. The past programme experience will be used to develop a country-wide strategy. Links with DHS will be sought.

Operational research: Preparations for the national TB prevalence survey are underway, with rather slow progress and a financial gap has been identified. There is lack of an operational research strategy and capacity for implementing it at all levels. Operational research is not done systematically in problem areas making it difficult to improve services since there is not much researched information available on obstacles to optimal service delivery.

4.3.2. Health System

Describe the main weaknesses of and/or gaps in the health system that affect tuberculosis outcomes.

The description can include discussion of:

- *issues that are common to HIV, tuberculosis and malaria programming and service delivery; and*
- *issues that are relevant to the health system and tuberculosis outcomes (e.g.: PAL services), but perhaps not also malaria and tuberculosis programming and service delivery.*

Human Resource

There is a critical shortage of Human Resource for Health (HRH) at all levels. According to the Primary Health Services Development Program 2007-2012 (PHSDP), Tanzania's health system operates with a third of the required workforce particularly clinicians, nurses, laboratory and pharmaceutical technicians. There is also inadequate training and refresher courses for the workforce.

Practical Approach to Lung Health

The health care delivery system in Tanzania has limited attention for a syndromic approach to lung health, resulting in non-comprehensive care. Currently a systematic approach to management of patients with all types of respiratory signs and symptoms has not been introduced and hence there is no consistency with practicing clinicians. As such, TB suspects are often inappropriately managed and often misdiagnosed and get a relatively late diagnosis. In the era of HIV, any person with cough must be considered to have tuberculosis, until proven otherwise.

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Infrastructure

Current health facilities were not built to cater for emerging challenges of MDR TB and TB/HIV services. TB clinics in many places do not have enough space and privacy to provide collaborative TB/HIV activities and did not take into consideration infection control plans. For the health sector as a whole, there is lack of effective diagnostic tools, equipment and supplies for delivering ATM services at service delivery points.

Health Information systems

Integration of the current TB recording and reporting system and use of information by management is constrained by weakly functioning HMIS; specifically completeness and timeliness of reporting, feedback mechanism, age and sex disaggregation of data, coordination, linkage of databases, development of analytical capacity, and optimizing utilization in decision making are weaknesses within HMIS at all levels.

Procurement and Supply Management

Many RHMTs and CHMTs do not play their supervisory role in anti-TB drugs supply and management. In many primary health care facilities, but also in regional stores dealing with anti-TB drugs ordering and supply to districts, the routine store management principles are inadequately adhered to, even among skilled personnel trained in drug safety and storage. Some of the existing district pharmacy stores are not spacious enough to accommodate the current supplies of anti-TB drugs.

Leadership and governance

This is not strong enough, from national to health facility level. While the programme has most SOPs summarized in the National NTLP manual and other guidelines, the adherence to procedures is lagging behind expectation. National and regional level TB programme staff has followed international management training courses, but they are often unable to apply the newly acquired skills sufficiently, which hampers the timely implementation of planned activities, while funds are available. Available tools for staff performance assessment (OPRAS) are hardly ever used at all levels; non-performance has often no consequences.

4.3.3. Efforts to resolve health system weaknesses and gaps

Describe what is being done, and by whom, to respond to health system weaknesses and gaps that affect tuberculosis outcomes.

Tanzania's strategic response to communicable diseases (and non communicable diseases) particularly ATM and larger health systems weaknesses is articulated through Strategy 8 in the Health Sector Strategic Plan III (2009 - 2015) and operationalized through annual MoHSW MTEFs, Comprehensive Council Health Plans (CCHPs) and Regional Health Plans.

Human Resource: Enrolment in health training institutions was only 16% (1,013) of the MoHSW's goal of 6,450 prior to 2007. The government increased this number by 31% to reach 3,000 trainees. This intervention effectively reduced the gap from a shortfall of 84% to 53%. However, a number of challenges such as poor infrastructure, inadequate facilities, learning and teaching materials, and a shortage of tutors have not been addressed adequately.

The distribution of health care workers (HCW) across the country heavily favours urban areas, with low HCW densities in rural, hard-to-reach areas. To address this challenge, the GoT launched an Emergency Hiring Programme (EHP) in 2007 in collaboration with the Benjamin Mkapa AIDS Foundation. To date, the EHP has experienced above 80% retention rate of the 176 individuals recruited and deployed to previously underserved locations. Moreover, Global Fund round 3 and 6, PEPFAR and PATH has deployed TB/HIV officers in 107 districts in the country. Through Global Fund round 6, human resources have been recruited at the central and peripheral levels to support TB/HIV, ACSM and MDR TB management in the country.

Practical Approach to Lung health (PAL): Under the guidance of the MoHSW and with WHO support, a syndromic approach to health service delivery will be introduced for a number of aspects of general medicine. The generic WHO materials for IMCI and IMAI were adapted to the Tanzanian situation, notably in the field of HIV/AIDS. IMCI is up and running country-wide. Training courses for

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IMAI are planned. Further adaptation to include PAL will go a long way to address the shortcomings, raise the suspicion index and increase case detection for TB, while improving lung health care in general.

Infrastructure: The Government of Tanzania and Global Fund round 6 have set up the MDR TB complex in Kibong'oto National TB hospital to accommodate the inception of MDR TB management in the country. Other partners include ABBOTT foundation who are supporting the MoHSW in strengthening the infrastructure of regional laboratories in the country. PEPFAR are supporting the construction of a Zonal TB laboratory in Mbeya referral hospital and new TB/HIV clinics to have special emphasis on privacy, infection control and ARVs provision in the same setting in selected high burden districts in the country. Through the previous Joint Rehabilitation Fund of the Health Basket and now the Health Window of the Local Government Development Grant (LGDG), significant work has been done to repair, rehabilitate, expand and in places, reconstruct health facilities. The aim of the Government through the Primary Health Service Development Program is to have a dispensary in each village and a Health Centre in each ward.

Health Management Information System (HMIS): The MoHSW has embarked on a plan to review and further develop a comprehensive HMIS system which is being supported by a consortium including Ifakara Health Research and Development Centre, University of Dar es Salaam and the University of Oslo. The HMIS system now covers all health facilities in the country, including FBO and private sector providers. The nomination of focal points in each Regional and Council Health Management Team (RHMT and CHMT) has also led to some improvement in data quality and timeliness of transmission and the use of data at the district level for planning. Gender specific data are collected on a routine basis. NTLP started using an Electronic Tuberculosis Register, ETR.Net, in 2007 to ensure effective surveillance of TB and TB/HIV patients across the country. The use of this software has effectively improved data collection, compilation and management of data. GFR3&6, PATH and CDC supported districts are currently using this software in recording and reporting TB/HIV data following training of health workers in the implementing districts. Installation of computers and software is still required for effective implementation of ETR.net in 20 remaining districts and replacing computers in all 21 regions.

Procurement and Supply Management: The Procurement Management Unit of the MoHSW undertakes all procurement centrally except drugs and medical supplies, which is managed by MSD. For years NTLP has worked closely with MSD regarding procurement and supply of anti-TB drugs and other supplies for TB control. In 2008 the MoHSW deployed a pharmacist to the NTLP CU to oversee NTLP drugs and supplies management. NTLP invites MSD and regional staff (all RTLCs, selected RMOs, pharmacists) to attend its bi-annual meetings, to ensure awareness of the roles they have to play in the oversight of TB control activities. One of the primary roles of the RTLCs and DTLCs is to visit pharmacy stores on a regular basis to ensure adequate and timely ordering, storage and distribution of drugs.

Leadership and governance: Health Sector Reform follows and sometimes leads the Civil Service Reform, which is underway since more than ten years. Improving staff management is part of the efforts to improve service delivery, including a performance assessment (OPRAS).

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4.4. Round 9 Priorities

Clarified Section 4.4

Complete the tables below on a program coverage basis (and not financial data) for **three to six areas** identified by the applicant as priority interventions for this proposal. Ensure that the choice of priorities is consistent with the current tuberculosis epidemiology and identified weaknesses and gaps from s.4.2.2 and 4.3.

Note: All health systems strengthening needs that are most effectively responded to on a tuberculosis disease program basis, and which are important areas of work in this proposal, should also be included here.

Priority No:1	Pursue high quality DOTS expansion	Historical		Current		Country targets			
Indicator name	Case detection rate (Number and percentage of new smear positive TB patients reported to NTLP among new smear positive patients estimated to occur in Tanzania per year)	2007	2008	2009	2010	2011	2012	2013	2014
A: Country target (from annual plans where these exist)		33600	34300	25480	23950	28800	30550	32300	33040
B: Extent of need already planned to be met under other programs?		22560	23520	24500	25480	26400	26790	28500	30680
C: Expected annual gap in achieving plans				980	1470	2400	3760	3800	2360
D: Round 9 proposal contribution to total need		(e.g., can be equal to or less than full gap)			1470	2400	3760	3800	2360

Priority No:2	Strengthen collaborative TB/HIV activities	Historical		Current		Country targets			
Indicator name	Number and percentage of TB patients who had a HIV test result recorded into the TB register among the total number of registered TB patients	2007	2008	2009	2010	2011	2012	2013	2014
A: Country target (from annual plans where these exist)		37566	49102	51567	56054	60080	63722	66030	67388
B: Extent of need already planned to be met under other programs		31305	47843	35452	37589	40054	43599	47651	51572
C: Expected annual gap in achieving plans				16115	18455	20027	20123	18379	15816
D: Round 9 proposal contribution to total need		(e.g., can be equal to or less than full gap)			18455	20027	20123	18379	15816

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Priority No:3	Strengthen management of resistant tuberculosis	Historical		Current		Country targets			
Indicator name	<i>Number and percentage laboratory-confirmed MDR-TB patients enrolled in second line anti-TB treatment among the total number of estimated MDR-TB patients</i>	2007	2008	2009	2010	2011	2012	2013	2014
A: Country target <i>(from annual plans where these exist)</i>		362	367	372	377	382	387	398	403
B: Extent of need already planned to be met under other programs		20	20	20	20	20	20	0	0
C: Expected annual gap in achieving plans				352	357	362	367	398	403
D: Round 9 proposal contribution to total need		<i>(e.g., can be equal to or less than full gap)</i>			20	40	40	80	100

→ If there are six priority areas, copy the table above once more.

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4.5. Implementation strategy

4.5.1. Round 9 interventions

Explain: (i) who will be undertaking each area of activity (which Principal Recipient, which Sub-Recipient or other implementer); and (ii) the targeted population(s). *Ensure that the explanation follows the order of each objective, program work area (or, "service delivery area (SDA)"), activities and indicator in the 'Performance Framework' (Attachment A). The Global Fund recommends that the work plan and budget follow this same order.*

Where there are planned activities that benefit the health system that can easily be included in the tuberculosis program description (because they predominantly contribute to tuberculosis outcomes), include them in this section only of the Round 9 proposal.

Note: If there are other activities that benefit, together, HIV, tuberculosis and malaria outcomes (and health outcomes beyond the three diseases), and these are not easily included in a 'disease program' strategy, they can be included in s.4B in one disease proposal in Round 9. The applicant will need to decide which disease to include s.4B (but only once). → Refer to the [Round 9 Guidelines](#) (s.4.5.1.) for information on this choice.

4.5 Implementation strategy

BETWEEN 4 to 8 PAGES

The Principal Recipient of the GFR 9 grant will be the Ministry of Finance and Economic Affairs, with the NTLP of the Ministry of Health and Social Welfare (MoHSW) and Prime Ministers' Office Regional Administration and Local Government (PMORALG) as the lead sub-recipients. MoHSW - NTLP will also be responsible for technical oversight. In line with the Stop TB Strategy, the NTLP has decided to focus its programme for the next five years on improving and expanding its ongoing activities with the following goal and the corresponding seven objectives.

Goal:

To achieve universal access to quality TB care with gender mainstreaming

Impact Indicator

Detection of TB cases (all forms) of 250 per 100,000 by the fifth year of this program.

Objective 1: Pursue high quality DOTS expansion with special focus on children and marginalized

The end term review of five years (2004-2009) NTLP strategic plan has recently been completed. The review report on the findings has indicated a number of weak areas in DOTS services provision in the country. Experience from previous supervision reports has documented weaknesses in supervision of the districts by the regional health management teams. There are challenges with regard to technical capacity and limited workforce. Regions and districts need to be empowered to undertake supervision of global fund services. Central Unit supervision and coordination under the MoHSW also needs to be strengthened.

The implementation of interventions under this objective will put much more emphasis on strengthening the capacity of the districts to implement quality DOTS.

While children below the age of 15 years old in Tanzania account for 40% of the total population, most of clinicians do not usually suspect TB among the children and only few are reported to have TB. Diagnosis of tuberculosis in children is challenging as most of them cannot produce sputum and most of the times present with non specific symptoms. This programme will build on the specific strategies to improve this important area. Improvement of diagnostic services by strengthening external quality assessment, scaling up introduction of new technologies and strengthening drug management in PHC facilities. Capacity of the MoHSW to use data to forecast drug and supply needs and rationally manage drug procurement and logistics will continue to be strengthened through short term training for relevant staff in this field. The strengthening of the management of supply chain and improvement of storage facilities will also be emphasized under HSS. The district quarterly TB reports have been adapted to allow the capturing of data on all forms of TB among 0-14 years old. Three service delivery areas with corresponding activities

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have been formulated to achieve the objective.

SDA 1.1 High quality DOTS

Indicator 1.1.1 Case detection rate (Number and percentage of new smear positive TB patients reported to NTLP among new smear positive patients estimated to occur in Tanzania per year).

Activity 1.1.1.1 Develop SOPs and Job aids on quality DOTS for health care providers in all DOTS centres. NTLP-CU and partners will organize a 5 day workshop involving 30 participants SOPs and job aids are at present not available in health facilities for quick reference and guidance. However, various guidelines and TB control related operational documents are existing, but not always accessible.

Activity 1.1.1.2 Print and distribute 11 sets of SOPs and Job aids on quality DOTS to each of the 5,000 health facilities implementing DOTS.

Activity 1.1.1.3 Train 444 health care providers from two remote health facilities in each of the 111 rural districts on sputum collection and fixing of smears at specimen collection points. This activity will be implemented to scale up Fidelis project innovations and the experiences gained by PATH: smear fixing by low cadre trained health workers is practically possible in remote areas. Regional and district lab technologists will conduct these training of health care providers working in facilities where AFB microscopy is not available.

Activity 1.1.1.4 Support 111 rural districts to transfer fixed sputum smear from remote specimen collection points to the nearest diagnostic centres. The fixed smears will be transferred to the nearest diagnostic centre by a responsible person at each of the sputum collection/fixing point.

Activity 1.1.1.5 Develop/Adopt paediatric TB training manual for health care providers during a six day workshop involving 12 participants. NTLP will engage a local consultant to guide the process. The use of the manual will raise the index of TB suspicion among health workers.

Activity 1.1.1.6 Train trainers regarding paediatric TB in 8 zones (zonal training centres). This activity will be outsourced to a consulting firm, 7 days workshop for 32 participants.

Activity 1.1.1.7 Orient 40 tutors of allied health training institutions to include pediatric TB and ISTC in their curricula. The implementation of this two days activity will be coordinated by NTLP.

Indicator 1.1.2 Treatment success rate (Number and percentage of new smear positive TB patients successfully treated among the new smear positive TB patients registered per year)

Activity 1.1.2.1 Provide transport fare support to TB patients living more than 5 km from DOTS centre: Adherence to treatment is very important for treatment success. Many patients living more than 5 km from the DOTS centre fail to reach the health facilities due to prohibitive transport costs. Both the treatment supporter and the patient are targeted. CHMTs will be facilitated to undertake this activity.

Activity 1.1.2.2 Conduct drug and other medical supplies stock taking and audit. NTLP at national level will team up with pharmaceutical supply, internal audit and finance department to perform regular monitoring of anti-TB drugs and supplies to all regions. Regions will be supported to conduct similar anti-TB drugs and supplies verification to all districts, while each district will verify drug management at health facility level.

Activity 1.1.2.3 Procure first line drugs for 62,000 patients annually. Anti-TB drugs will be purchased annually commencing 2012 when the current facility through GDF and WHO expires.

Activity 1.1.2.4 Rehabilitate/Refurbish existing district pharmacy stores in 102 districts. Rehabilitate ceiling, floor, walls, paint, partition, grill door & windows, shelves air condition and refrigerator. The refurbishments conducted during the implementation of GFR 6 in 31 districts will be scaled up to other underserved district pharmacy warehouses. District local authorities will implement this activity.

SDA 1.2 Improving diagnosis

Indicator 1.2.1 Number and percentage of new smear-positive pulmonary TB patients reported quarterly among TB suspects investigated.

Activity 1.2.1.1 Procure for 133 districts, laboratory equipment, supplies, reagents and new diagnostic tools for quality DOTS services. NTLP through MoHSW procurement unit will undertake the activity to improve quality TB diagnostic services in the country.

Activity 1.2.1.2 Request technical assistance (TA) to support introduction of new diagnostic technologies.

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Diagnosis of smear negative TB particularly in patients co-infected with HIV requires highly sophisticated technologies and the availability of TA will be a necessity.

Activity 1.2.1.3 Support X-ray costs at US\$ 10 for an estimated 18,000 AFB negative TB suspects in all districts. RHMTs and CHMTs will manage the process of implementation to ensure all TB suspects requiring x-ray services do not miss it. At the moment, a TB suspect needs to pay US\$ 4 or up to more than US\$ 15 for a chest x-ray picture in public and private health facilities respectively.

Activity 1.2.1.4 Train 20 laboratory staff from each of 21 regions on LED (light emitting diode) microscope and external quality assurance (EQA).

Activity 1.2.1.5 Conduct quarterly follow up and supportive supervision of districts implementing EQA. Regular monitoring and mentoring of front line health care providers will be conducted by RHMTs and CHMTs.

Activity 1.2.1.6 Conduct refresher training of 20 laboratory staff from each of 21 regions on smear microscopy including EQA. Annual refresher training was ongoing and funded by the programme. However, the funding for this training and regular supervision and EQA of peripheral labs has not been available for three years. Zonal TB Reference labs will organize these training courses.

SDA 1.3 Monitoring and evaluation

Indicator 1.3.1 Number and percentage of districts submitting timely reports

Activity 1.3.1.1 Train 420 health workers on the updated data collection tools and electronic TB register. NTLP in collaboration with other partners will train all regional, district TB and TB/HIV coordinators, HMIS officers and front line health care workers.

Activity 1.3.1.2 Procure and distribute computers and accessories to 20 districts and replace in 21 regions for ETR. NTLP through the MoHSW procurement unit will ensure quality products to all districts to improve data collection, analysis and further local use for better TB control planning.

Activity 1.3.1.3 Conduct supportive supervision at all levels: Regular monitoring and mentoring of front line health care providers will be conducted by 5 PMU-CU NTLP officers will supervise 12 regions each for 7 days once a year, RHMT teams will visit each of the districts twice a year for two days. Three CHMT members will visit health facilities for one day once a month, in addition to the routine CHMT matrix supervision currently funded by the Health Basket.

Activity 1.3.1.4 Procure 40 vehicles and 100 motorcycles for supervision in five years. NTLP transport system has mainly been supported by GLRA. There are 26 4WD vehicles for regions and four 4WD vehicles at NTLP-CU scheduled for replacement after every three to four years at a rate of eight vehicles a year. Among the 265 motorcycles available in the NTLP district network for supervision, replacement is required at a rate of 20 per year. Procurement will be governed by the Public Procurement Act.

Activity 1.3.1.5 Support Central Unit NTLP administrative cost. These include salaries for two accountants and top-up allowances for 42 staff; running costs for four NTLP CU vehicles; and office costs like telephone, internet, stationery, postage and computer accessories.

Activity 1.3.1.6 Conduct mid and end term evaluation of GFR-9 programme activities with participation of internal and external consultants: This will be done at the third and fifth year of implementation to assess achievement, quality, effectiveness and coverage.

Objective 2: To strengthen collaborative TB/HIV activities with special focus on congregate settings.

Collaborative TB/HIV activities were introduced in the country in 2005. Though there has been rapid scale up of these activities, they are concentrated mainly in the urban areas. Most rural health facilities are yet to embark on TB/HIV activities and so are those in congregate settings. A good number of health workers need training on these activities which are increasingly in demand. It has also been reported that there is insufficient prescription of Co-trimoxazole to TB/HIV co-infected patients and low ART initiation. This proposal intends to expand TB/HIV activities to address these shortfalls and also introduce TB infection control measures in health facilities that have infective TB patients.

SDA 2.1 TB/HIV

Indicator 2.1.1 Number and percentage of TB patients who had a HIV test result recorded into the TB

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register among the total number of registered TB patients

Activity 2.1.1.1 Conduct TB/HIV needs assessment in prisons, police and military forces: This will be done during the first year in a total of 32 health facilities – 15 prisons, 11 police and six military health facilities in order to identify their capacity (human resource, infrastructure, and existing services) including the political willingness and readiness to implement TB/HIV services. Based on the established criteria those that have capacity and need strengthening will be identified.

Activity 2.1.1.2 Support region and district levels to conduct joint planning for TB and TB/HIV activities: A joint workshop will be conducted to RHMTs/CHMTs, TB and HIV/AIDS coordinators at the regional level to introduce them on grant terms and conditions of GFR-9 and activities to be planned for implementation at the region and Council levels.

Activity 2.1.1.3 Upgrade, renovate and refurbish TB/HIV facilities including prisons, army forces and the mining sector: A total of 562 health facilities (four in each of the 133 councils, 11 police, 13 prison and six military) will be upgraded and refurbished in line with TB infection control to provide quality TB/HIV services: well-ventilated waiting space, consultation and counseling rooms for PITC, phlebotomy room, confidential filing room. Most of the TB clinics have small debilitated rooms, so minor expansions will be needed. In councils where there are mines, priority will be given to TB clinics serving the mining community.

Activity 2.1.1.4 Review, print and distribute collaborative TB/HIV training package to incorporate paediatric TB/HIV and TB infection control measures: Paediatric TB/HIV notification is minimal and currently there is low implementation of TB infection control measures. The current package will be reviewed to address these shortfalls. The revised package will be printed and distributed to all 133 councils.

Indicator 2.1.2 Number and percentage of HIV-positive TB patients who start on or continue previously initiated antiretroviral therapy, during or at the end of TB treatment, among all HIV-positive TB patients registered quarterly.

Activity 2.1.2.1 Conduct TB/HIV training to health care providers in 133 districts including prisons, army forces and the mining sector: A total of 3,325 health staff from all categories both public and private facilities will undergo a four day training course in their districts on collaborative TB/HIV activities including PITC, pediatric TB/HIV, performance improvement approach and TB infection control measures to build their capacity to manage TB/HIV co-infected patients. Training is provided by trainers from the established pool, in collaboration with zonal training centres.

Activity 2.1.2.2 Conduct refresher training of health care providers on TB/HIV activities including PITC and TB infection control measures in 133 Councils: This activity aims to strengthen human capacity and fill the identified gaps in knowledge and skills. A total of 3,325 health staff from both public and private will undergo this training.

Activity 2.1.2.3 Recruit and deploy 25 district TB/HIV officers. Already 106 TB/HIV officers are in place through various supports, namely GFR6 36, GFR3 11, CDC 47 and PATH 12. TB/HIV officers will be deployed to coordinate TB/HIV activities at district level, one in each of 25 remaining districts. Their initial training will be incorporated in available TB/HIV training of health workers. This staff will require annual salaries for a period of five years. Their future salaries will be integrated in the district health plans for sustainability.

Activity 2.1.2.4 Procure and distribute a vehicle and motorcycles for district TB/HIV officers: One vehicle is needed for central administration. 25 motorcycles will be procured and distributed to each district TB/HIV officer for transport assistance during supportive supervisory visits.

Activity 2.1.2.5 Provide enablers to improve working conditions and incentives to TB/HIV staff in 133 Councils: Boots, umbrellas, raincoats, air time, and other relevant commodities will be provided.

Activity 2.1.2.6 Provide quarterly administrative costs, vehicle and motorcycles running costs for central level and district TB/HIV officers: The funds allocated in this activity will be used for administrative costs at the central and district levels such as stationery, communication, fuel for logistic support and insurance; There is no assurance of continuity of funds from current partners beyond 2011.

Activity 2.1.2.7 Support RTLCC, RACC, DTLC, TB/HIV officer and DACC to participate in CCHP: This will be for attending the district health plans meeting in 97 councils to incorporate TB, HIV and TB/HIV activities into CCHP for sustainability. TB and HIV/AIDS coordinators from the remaining 36 district are

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being supported by GFR-6.

Activity 2.1.2.8 Facilitate quarterly coordinating committee meetings at national, regional and district levels: Sustainability of the existing TB/HIV coordinating bodies is a necessity to ensure more effective collaboration between TB and HIV/AIDS programs at all levels. The bodies at the national, in 21 regions and 133 councils will meet quarterly to appraise technical progress, and financial reports and implementation of the annual plan at the respective levels.

Activity 2.1.2.9 Facilitate dissemination of TB and TB/HIV information in local and international meetings: Participation in these meetings is for exchange of information, sharing of best practices, challenges, and the way forward so as to improve services and will involve participation of four people to attend three annual international conferences. Support of local meetings is for the annual TB Coordinators meeting/ annual TB/HIV implementers' meeting.

Activity 2.1.2.10 To renovate and refurbish medical wards: The funds allocated in this activity, will be used to renovate medical wards in 90 district hospitals, to set up isolation room/wards for infectious TB patients who will meet criteria for admission. This is in line with WHO recommendations and Tanzania National policy for collaborative TB/HIV activities.

Indicator 2.1.3 Number and percentage of HIV-positive TB patients who receive at least one dose of Co-trimoxazole preventive therapy during TB treatment among all HIV positive TB patients registered quarterly.

Activity 2.1.3.1 Procure, distribute and provide Co-trimoxazole for TB/HIV co-infected patients: As part of the WHO recommended package of care to HIV/AIDS patients, Co-trimoxazole will be provided to 30,000 TB/HIV co-infected patients to reduce morbidity and mortality by preventing opportunistic infections.

Activity 2.1.3.2 Procure and distribute Dapsone tablets for people living with HIV who are allergic to Co-trimoxazole: Dapsone will be provided to an estimated 10,000 persons (2.5% of 30,000 TB/HIV and 400,000 PLHA allergic to Co-trimoxazole).

Objective 3: To strengthen management of MDR-TB

The surveillance for MDR TB in Tanzania is weak with few of the estimated patients being diagnosed. Health workers will need to understand the importance of regular surveillance including safe transport and handling of sputum, hence the need for training. This will go hand in hand with expanding the capacity to perform sputum culture in the country. External quality assurance is done biannually in collaboration with the supra-national laboratory at Antwerp, Belgium. Tanzania will start treating MDR TB patients with second line drugs in the final half of 2009, as drugs are expected to arrive in July. The recently renovated Kibong'oto Hospital is not able to handle all the MDR TB patients who will require treatment. This proposal therefore plans to establish a satellite MDR TB unit in Dar es Salaam, the region that diagnoses about half of all MDR TB patients in Tanzania. As stipulated in HSSP III, regional hospitals will be updated to function as referral centres for PHC facilities within their catchment areas. It is therefore proposed that some selected regional hospitals will be strengthened to perform line probe assay for DST. The focus of expanding MDR-TB services management to community will also be emphasized

SDA 3.1 Multidrug resistant TB (MDR-TB)

Indicator 3.1.1 Number and percentage of laboratory-confirmed MDR-TB patients among new and retreatment smear-positive TB patients as a percentage of the estimated number of MDR-TB patients among new and retreatment smear-positive TB patients.

Activity 3.1.1.1 Facilitate transportation of up to 10,000 specimens annually from districts to zonal laboratories, and positive cultures from Zonal laboratories to CTRL.

Activity 3.1.1.2 Strengthen four regional laboratories to perform TB cultures: at present Tanzania has four functional TB culture laboratories, which is not adequate according to WHO guidelines, 1 culture TB lab to serve 5 million people.

Activity 3.1.1.3 Procure and provide appropriate transport containers and supportive materials for transporting sputum specimens: 12,000 Specialized sputum containers per year are needed for safe collection and transportation of sputum specimens from TB diagnostic centers to culture laboratory. Additional packaging materials are necessary to prevent breakage and contamination of the containers.

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Activity 3.1.1.4 Provide incentives to laboratory staff involved in management and administration of sputum for culture and DST: laboratory staff at seven zonal laboratories and one central reference laboratory will be in close contact with high risk TB specimen. Some incentives will empower and motivate them to perform their tasks effectively.

Activity 3.1.1.5 Support laboratory staff to attend special training on DST and line probe assays: it is planned to train 10 laboratory staff from central and zonal TB laboratories

Activity 3.1.1.6 Introduce line probe assays at CTRL, 3 zonal TB reference laboratories, Kibong'oto TB hospital and 18 regional hospitals as a new technology for rapid diagnosis of MDR-TB: Line probe assays are currently one of the best techniques for rapid diagnosis of MDR TB, it is proposed that to start with, NTLP will initially lease a line probe assays facility for CTRL.

Indicator 3.1.2 Number and percentage laboratory-confirmed MDR-TB patients enrolled in second line anti-TB treatment

Activity 3.1.2.1 Procure DOTS Plus drugs through Green Light Committee (GLC): The allocated funds will be used to procure second line Anti-TB drugs for 60 patients (40 plus 20 as buffer) annually.

Activity 3.1.2.2 Identify and upgrade/refurbish a satellite unit in Dar es Salaam: Through routine surveillance procedures, Dar es Salaam notifies most of the MDR TB patients. Therefore it is important to bring the service closer to the patients.

Activity 3.1.2.3 Establish MDR-TB satellite unit in Dar es Salaam. In order to compliment the workload of the existing staff in the implementation of DOTS-Plus program, there shall be recruitment and deployment of 3 clinicians, 4 nurses, 2 social workers, 2 psychiatric nurses, 1 nutritionist, 2 laboratory staff and 1 driver to strengthen the workforce at the MDR-TB satellite unit.

Activity 3.1.2.4: Train 30 staff through an international MDR-TB training course to manage patients at the MDR TB satellite unit in Dar es Salaam. This will be international training for six staff from different cadre each year.

Activity 3.1.2.5 Train healthcare workers on the management and care of MDR(X) TB patients: Training will involve 528 frontline health workers from the districts who will be involved in management of MDR TB patients in the continuation phase of treatment.

Activity 3.1.2.6 Support referral of MDR TB patients from 133 districts for MDR TB management: this will involve transportation of patients. Patients who die at the hospital will also be transported together with an escort for burial at domicile places.

Activity 3.1.2.7 Procure and maintain one 4WD vehicle for logistical support at the Dar es Salaam MDR TB satellite unit.

Activity 3.1.2.8 Provide special risk allowances to care providers managing MDR-TB patients in the Dar es Salaam satellite unit: To ensure smooth running of the activities in the MDR-TB hospital, a monthly risk allowance will be paid to health care workers who will full time work in the management of the MDR-TB patients. Staff (health workers and support staff) will be paid monthly risk allowance for the period of five years.

Activity 3.1.2.9 Provide food and essential requirements (soaps, linens, fruits, milk and entertainments) for MDR TB patients: Patients will be admitted in the specialized MDR-TB satellite clinic for prolonged periods thus the need for socio-upkeep assistance.

Indicator 3.1.3 Number and percentage of Laboratory-confirmed MDR-TB patients successfully treated (cured plus completed treatment) among those enrolled in second line anti-TB treatment at 24 months and if necessary 36 months.

Activity 3.1.3.1 Procure laboratory equipment and supplies for monitoring side effects to second line drugs: Laboratory equipment for measuring electrolytes and TSH will be procured to monitor life threatening side effects of second line drugs.

Activity 3.1.3.2 Seek Technical assistance (TA) to support capacity building in the management of MDR TB (including WHO, IUATLD, KNCV, UCSF, CDC):

Activity 3.1.3.3 Conduct special supportive supervision to the MDR-TB referral units: NTLP together with

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visiting TAs will conduct technical supportive supervision and quality control to the MDR-TB referral units twice per year.

Objective 4: Strengthen health systems capacity with emphasis on the Practical Approach to Lung Health (PAL)

According to WHO Tanzania detects only half the estimated number of smear positive TB cases. There seems to be inadequate investigation of patients with respiratory problems for tuberculosis. There is weakness in the health systems hence most patients with respiratory complaints go through the system unchecked with no suspicion of TB. This proposal requests to embark on the roll out of Practical Approach to Lung Health as a means of streamlining proper investigations for respiratory diseases and appropriate referral to the TB Unit. This introduction, that will compliment IMCI and IMAI, will start at tertiary centres and will gradually be disseminated throughout the country in a phased manner.

SDA 4.1 Practical Approach to Lung Health (PAL)

Indicator 4.1.1: Number and percentage of health facilities implementing PAL among the total number of health facilities.

Activity 4.1.1.1 Conduct training to relevant professional staff at tertiary hospitals (Muhimbili National Hospital, Bugando, Mbeya, and Kilimanjaro Christian Medical Centre) to familiarize with PAL manual, review SOPs and guidelines and to devise an appropriate strategy for institutionalization of PAL in a phased approach. Training shall be facilitated by Zonal Training Centers (ZTCs); at first will require 4 experts (2 external and 2 internal) for 7 days duration targeting 2 TOTs from each zonal training centre.

Activity 4.1.1.2 Institutionalize PAL at the four tertiary hospitals. Within each tertiary institution, there shall be a training of staff from various departments using zonal TOTs for duration of 4 days involving up to 10 health workers per each tertiary institution. The trainings will be coordinated by ZTCs.

Activity 4.1.1.3 Purchase distribute and provide for maintenance of essential equipment appropriate for operationalization of PAL at first-level health facilities, district and regional referral outpatient services or laboratories and at second referral level as per WHO guidelines. Medical store department will procure and distribute the equipment (stethoscopes, examination torches, otoscopes, pulse oximeter, aspiration equipment, instruments for pleural biopsy and drainage,) to 40 hospitals in the country.

Activity 4.1.1.4 Preparation of appropriate IEC material for distribution to the operating sites. This will require design, pre-testing, printing, of leaflets and posters, design of messages for public advertising boards, and mass media (newspapers, radio and TV). The production of advocacy materials will be done by the consulting firm. The development of advocacy materials and printings will be done through appropriate tendering board. Materials include 15,000 leaflets, 5,000 posters of various sizes and 75 billboards. Mass media activities will include 100 adverts, TV and radio spots for each of 4 selected newspapers, TV and radio stations per year.

Activity 4.1.1.5 Extend PAL to initial 15 selected hospitals (district/municipal hospitals and private hospitals). Train 5 health workers per hospital for 4 days. The relevant consulting firm in collaboration with ZTCs will be contracted to coordinate this activity.

Activity 4.1.1.6 Scale up of PAL to all 21 regional hospitals with focus on local institutionalization and their role in extending PAL to primary health facilities and FBO and private health facilities. Three days training involving up to 5 health workers per hospital. This activity will be conducted by the respective ZTCs in each region.

Activity 4.1.1.7 Support update of curricula in clinical and nursing care training institutions to address the PAL approach. Professional Councils (Medical, Nursing), Tutors from ZTCs and selected allied health schools and colleges shall undertake 2 curriculum update workshops each lasting one week conducted at ZTCs. Total participants per workshop shall be 20 with 3 facilitators (1 external consultant and 2 local – zonal TOTs). ZTCs will undertake this activity

Activity 4.1.1.8 Support training of teaching staff on PAL and early TB detection. 4 days training of tutors

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per session, 20 participants and three times each year. Zonal training centres in collaboration with MoHSW-human resource department will implement the activity.

Activity 4.1.1.9 Conduct a survey to establish epidemiological and social burden on respiratory diseases. One local expert shall be engaged to conduct the survey in collaboration with the National Bureau of Statistics. NTLP will organize the implementation of this activity.

Activity 4.1.1.10 Support development and dissemination of PAL practice brochures to increase knowledge on early detection of TB among community, PLHIVs and in-patient care providers. National identified expert panel (10 National experts) shall develop the brochure in a 5 day workshop. 100,000 copies of the brochure shall be printed and distributed to all districts. Printing firm will be obtained through tendering procedure of MoHSW and print materials distributed to 133 districts by MSD.

Objective 5: To empower people with TB and communities in TB care through ACSM, community participation and patient charter.

ACSM activities are used to achieve the six goals of Stop TB strategy. ACSM has been used successfully to address the challenges faced in TB control which include; increasing case detection and adherence, combating stigma and discrimination, empowering people affected by TB and mobilizing political commitment and resources for TB. Community members will be mobilized to be active participants in early case detection, retrieval of close contacts with TB symptoms as well as treatment and care of TB patients. This will increase the chances of patients to successfully complete treatment and reduce defaulter rates. Furthermore, active case finding, contact tracing and defaulter retrieval is made easy when the community is involved in the direct care of TB patients.

Both the two strategies above are proposed to be expanded so as to assist in reaching the goals in diagnosing and treating most TB patients.

SDA 5.1 Advocacy, communication and social mobilization

Indicator 5.1.1 Population with correct knowledge about TB (mode of transmission, symptoms, treatment and curability) (percentage)

Activity 5.1.1.1 Identify and assess stakeholders who could help to plan and implement ACSM for TB control. The first step in collaborative management is to identify and involve stakeholders. A three day meeting will be conducted at the national level with participation of 30 people to implement this activity.

Activity 5.1.1.2 Conduct KAP studies on community knowledge on TB: a series of studies will be done, during year one and year four. The studies will be conducted in 40 selected districts, 10 urban and 30 rural districts. Local research and public health institutions will be contracted to undertake this through a transparent selection process. The information obtained will be used to update the ACSM strategy and materials.

Activity 5.1.1.3 Support development of ACSM messages and concepts. NTLP and stakeholders will conduct a workshop at national level involving 20 experts in this field for 8 days. Messages to create public awareness on TB, MDR-TB, TB/HIV and Community TB Care will be developed and pre-tested. This activity will be implemented during the second year of this programme.

Activity 5.1.1.4 Print and disseminate ACSM messages and promotional materials. The developed ACSM messages will be printed in form of brochures, posters, billboards; and promotion materials such as calendars, pens, T-shirts, khangas and key-holders. Printing agency will be contracted to undertake this through a transparent selection process. These materials will be distributed to cover all the 133 districts.

Activity 5.1.1.5 Train media personnel on Tuberculosis. A two days training workshop on TB, MDR-TB, TB/HIV and Community TB Care will be conducted annually for 30 media personnel in each of the 21 regions. The RHMTs in collaboration with CSOs will organize the training. Experience gained from GFR6 on training media personnel on TB control has shown that involving the media can develop their interest so they become good advocates. However, they need to be reached and updated more regularly.

Activity 5.1.1.6 Facilitate development of, paid panel discussions, documentaries, radio and TV spots. The development of the spots that will be aired through TV and radio will be outsourced through a competitive bidding process to professional promotion agencies.

Activity 5.1.1.7 Broadcast ACSM messages through Radio and TV spots. TV and Radio will be used to

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air TB spots at prime hours for three days per week. Through these TV and radio air spots, people will get proper information on TB, MDR-TB, TB/HIV and CTBC and hence are expected to positively change their health seeking behaviour.

Activity 5.1.1.8 Collaborate with district authorities to train teachers and students on signs and symptoms of TB. School children are the best channel to raise awareness of TB at the community level. Ministry of Health and Social Welfare/NGOs in collaboration with the Ministry of Education will develop a training package on TB control during a one day meeting. 133 CHMTs will conduct three days training for school health programme teachers in their respective districts.

Activity 5.1.1.9 Orient key celebrities. NTLP and stakeholders will conduct one day orientation meeting with 20 celebrities on ACSM messages and concepts. Celebrities (footballers, musicians, beauty queens, TV and movie stars) will be oriented on TB disease and TB control strategies in order to obtain their involvement in TB control campaigns.

Activity 5.1.1.10 Facilitate formation of ex-TB patients clubs to participate on TB control strategies. MUKIKUTE is a club of former TB patients in Temeke District of Dar es Salaam, which is involved in community sensitization, supporting DOTS treatment, contact tracing through index cases, defaulter retrieval and provision of health education to TB patients. Using the MUKIKUTE experience, ex TB patient will be supported to form clubs, one club per region.

Activity 5.1.1.11 Sensitize influential people, community leaders and communities on tuberculosis in 97 districts. CHMTs in collaboration with CBOs will organize one day sensitization meetings on TB control for 60 people from ward level.

Activity 5.1.1.12 Sensitize Chief Executive Officers (CEOs) and Managing Directors (MDs) of major corporations in Tanzania on TB control. A one day seminar to 30 selected CEOs and MDs of public and private corporations will be conducted. This will make them aware of TB control issues and hence encourage training to their staff and also contribute financial support to additional ACSM activities e.g. billboards and posters.

Activity 5.1.1.13 Sensitize Members of Parliament on TB control strategies. A one day seminar to 370 MPs during parliamentary meetings will be done annually. Once these policy makers are well informed on TB control activities, they will enhance the political commitment of the government to TB control.

Activity 5.1.1.14 Support 133 districts to conduct road advocacy shows (twice every quarter) for TB control strategies. CHMTs will collaborate with local groups that will formulate appropriate messages about TB, TB/HIV, MDR TB and CTBC in form of songs, drama, comedy and other interesting actions.

Activity 5.1.1.15 Facilitate development of TV drama shows. Identify and support two selected local folk groups to develop five 15 minutes drama shows. The emphasis of the shows will be on issues concerning TB control, TB/HIV, MDR TB and CTBC.

Activity 5.1.1.16 Broadcast the developed drama shows on local TV stations. Each of five TV stations will air two of the 15 minutes drama shows twice per month.

SDA 5.2 Community TB care

Indicator 5.2.1. Number and percentage of new smear-positive TB patients managed or supervised by the community among the new smear positive TB patients reported to the national health authority (starting treatment in the national TB program).

Activity 5.2.1.1 Train health care providers on TB control and CTBC approach including health communication skills and patient empowerment: A three days workshop to train 1,940 health care providers on community-based DOT (20 from each district), will be conducted by CHMTs in collaboration with CSOs.

Activity 5.2.1.2 Orient traditional healers on signs and symptoms of tuberculosis and patient referral: A one day workshop will be conducted to orient traditional healers on signs and symptoms of tuberculosis and empower them to refer TB suspects to TB diagnostic centers for TB management, 25 traditional healers from each district, will be trained by CHMT/CBOs in 97 districts.

Activity 5.2.1.3 Orient owners and dispensers of private drug stores on signs and symptoms of tuberculosis and referral system: A one day training workshop on basic TB control with emphasis on

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signs and symptoms of TB will be conducted by CHMTs in collaboration with TFDA to 2,660 owners and dispensers of private drug stores in 133 districts. This will empower them to recognize TB suspects and refer them to DOTS centres since a majority of TB suspects delay diagnosis while seeking for self medication by purchasing cough syrups and antibiotics in private drug stores.

Indicator 5.2.2 Number and percentage of new smear positive TB patients successfully treated (cured plus completed treatment) among the new smear positive TB patients managed and supervised by the community.

Activity 5.2.2.1 Facilitate annual meetings with representatives of all CTBC stakeholders in the district including traditional healers: The CHMT will coordinate biannual meetings with other stakeholders including CBOs, FBOs, and NGOs to discuss challenges, experiences and the way forward. This is a one day meeting with participation of 30 people per district.

Activity 5.2.2.2 Conduct quarterly supportive supervision to communities: The supervision will be conducted by staff from 5,000 health facilities. Currently about 90% of TB patients receive DOTS through community based treatment supporters. It is important to conduct supportive supervision in the communities to monitor implementation of activities and visit the patients to monitor their progress.

Activity 5.2.2.3 Procure bicycles for DOTS health care workers: The bicycles will assist transport to the community for DOT supervision at community level and defaulter tracing especially for hard to reach areas. A total of 970 bicycles will be procured; 10 bicycles for each of 97 districts; the remaining districts are covered by GFR 6.

Activity 5.2.2.4 Develop SOPs and job aids on patients charter: A two day workshop involving 20 people to review and adapt the international patient charter will be conducted by NTLP in collaboration with CSOs.

Objective 6: Engage all health care providers in TB control in line with the International Standards for TB Care (ISTC)

Under this objective it is envisaged that engagement of public and private health facilities in TB control in line with ISTC. This will increase case detection and treatment success rate through

- Increasing the number of health facilities providing TB control in line with ISTC
- Increasing the number of pre- and in-service health care providers trained in TB control services in line with ISTC
- Increasing the number of private health facility owners in TB control services
- Increasing the number of lab staff from both public and private health facilities trained on LED Microscope and EQA
- Increasing the number of laboratories provided with LED microscopes to ensure quality diagnosis
- Increasing the number of refurbished private health facilities to provide TB control services.

The activities will be implemented under SDA 6.1 and indicator 6: 1.1:

SDA 6.1 All care providers (public-private mix and International standard for tuberculosis care)

Indicator 6.1.1 Number and percentage of new smear-positive TB patients managed or supervised by a specific type of health care providers among all TB patients reported to the national TB program.

Activity 6.1.1.1 Conduct assessment of private health facilities to identify gaps in implementing TB control strategies: three days assessment of private health facilities will be conducted by 2 people in each of 97 districts.

Activity 6.1.1.2 Orient private health facility owners on TB control strategies: One day orientation meeting on TB control strategies to 10 private health facilities owners per district will be conducted by CHMTs.

Indicator 6.1.2 Number and percentage of new smear-positive TB patients successfully treated (cured plus completed treatment) among those managed or treated by a specific type of health care providers

Activity 6.1.2.1. Facilitate attendance to quarterly public-private partnership meeting: A one day meeting involving stakeholders from both public and private health facilities will be conducted quarterly to share

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information and experiences in TB control services for five years.

Activity 6.1.2.2 Support refurbishment of private health facilities to provide quality DOTS: One private health facility per district in each year will get support for refurbishment. This will include partitioning, extension, provision of furniture and fittings.

Objective 7: Promote operational research to improve health service provision

In conducting operational research the program will increase the number of operational research studies and disseminated through a national or global TB M&E system. Local research and public health institutions will be contracted to undertake research through a transparent selection process after submitting respective protocols. The findings to be obtained from these studies will inform decision-making process for the better undertaking of TB control services in the country. In view of that, the following research areas will be implemented through the coordination of NTLP:

SDA 7.1 Operational research

Indicator 7.1.1 Number of operations research studies completed and results disseminated through a national or global TB M&E system.

Activity 7.1.1.1 Conduct drug resistance survey (DRS); The last DRS was conducted in 2007 and the findings reported need to be updated during the second year of this programme.

Activity 7.1.1.2 Conduct research on prevalence of TB/HIV in prisons. A researcher will be contracted to investigate the prevalence of TB/HIV in selected prisons with emphasis on challenges in diagnosis, treatment and care of affected prisoners, including issues pertaining to case holding.

Activity 7.1.1.3 Determine the best strategy for intensified TB case finding among PLHA and validate TB screening questionnaire. Despite introduction of screening tools in the CTCs, there is minimal use of this tool and literally no data from the CTC to report on the utilization of this tool. Investigations into the use of this tool, its perceived importance and obstacles that prevent its widespread use will be done.

Activity 7.1.1.4 Document and disseminate research findings. All operational research conducted will be documented, efforts made to disseminate the findings, together will appropriate incorporation into the health systems. This will be done through a consultant who will be identified.

4.5.2. Re-submission of Round 8 (or Round 7) proposal not recommended by the TRP

If relevant, describe adjustments made to the implementation plans and activities to take into account each of the 'weaknesses' identified in the 'TRP Review Form' in Round 8 (or, Round 7, if that was the last application applied for and not recommended for funding).

TWO PAGES MAXIMUM

Not applicable

4.5.3. Lessons learned from implementation experience

How do the implementation plans and activities described in 4.5.1 above draw on lessons learned from program implementation (whether Global Fund grants or otherwise)?

TWO PAGES MAXIMUM

The National Tuberculosis and Leprosy Programme (NTLP) has a clear vision, mission, strategies and targets set following the new global Stop TB Strategy and Global Plan to Stop TB 2006-2015. The mission of the programme is to provide high quality and effective interventions to control TB in the country with an ultimate goal of ensuring that all TB patients, including those co-infected with HIV and those with drug-resistant TB, benefit from universal access to high-quality diagnosis and patient-centered treatment focusing on equity, marginalized groups with gender mainstreaming. The NTLP also supports the development of new and effective tools to prevent, detect and treat TB to be reflected in the strategic plan 2010-2014.

The NTLP has made remarkable achievements that were clearly highlighted during the recently

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completed end term review of 2004-2009 NTLP strategic plan. The achievements made have positively contributed to the country's wider efforts to meet the vision 2025, the National Strategy for Growth and Poverty Reduction (NSGPR) goals and Millennium Development Goals. The NTLP is further committed to implement evidence-based interventions following global guidelines in an effort to assist the country in poverty reduction.

It is gratifying to note that the NTLP has achieved the WHO 2005 target of 85% treatment success rate and is still working hard in efforts to attain the 70% detection rate of infectious TB cases; putting emphasis on pediatric TB and smear negative TB diagnosis taking into account the challenges of TB diagnosis in the HIV era.

The NTLP is successfully implementing Community TB care and the Patient-Centered Treatment approach in selected districts under WHO, Novartis and GFR6 support. The results show that 90% of TB patients prefer community-based DOT versus facility-based DOT. The patients will be empowered to take a decision whether to have facility-based DOT or community-based DOT. The good experience gained will be scaled-up to cover the entire country. All TB stakeholders will be sensitized to actively participate in TB control and the country STOP TB Partnership will be formed for coordination purposes. The provision of quality community based supportive supervision remains a big challenge to sustain this success.

The country performance on TB/HIV Collaborative programme activities is good with almost 100% coverage though at different stages of implementation at district level. This success has been achieved through PEPFAR, GFR3&6 grant supports. This approach will be scaled-up for accessibility and equity purposes to cover selected facilities beyond the district hospitals in both public and private sector facilities and congregate settings. In order to effectively implement the TB/HIV services there is need for HSS for both human resource and infrastructural development.

One of the major limitations to increasing the number of TB patients in line with the global targets is limitations in the diagnosis of tuberculosis at the grass root level. In this regard, there is need to introduce new diagnostic technologies which are more sensitive – the LED microscope. This is anticipated to increase the detection rate from 50% up to over 70% even among TB suspects who are co-infected with HIV/AIDS. This will increase the number of new smear positive TB patients detected in the country.

Though the current MDR-TB prevalence is still low in the country (below 1%) due to effective DOTS strategy implementation, MDR-TB management is being introduced in Kibong'oto hospital Centre of Excellence under GFR6 grant. The challenge ahead is transportation of the MDR-TB patients to the Centre of Excellence and strengthening of the MDR-TB surveillance system. In this regard the MDR-TB surveillance system will be improved through strengthening of the three zonal TB reference laboratories and regional hospital laboratories to perform TB culture services and use of line probe assays for early detection of MDR-TB including ensuring external quality assurance in all laboratories in the country. The MDR-TB management will be scaled-up by introducing one MDR-TB satellite unit in Dar es Salaam city.

ACSM interventions have been successfully piloted in the country under Novartis Foundation and yielded good results. This is one of the interventions to be scaled-up to cover the entire country.

4.5.4. Enhancing social and gender equality

Explain how the overall strategy of this proposal will contribute to achieving equality in your country in respect of the provision of access to high quality, affordable and locally available tuberculosis diagnosis treatment and care and support services.

(If certain population groups face barriers to access, such as women and girls, adolescents, sexual minorities and other key affected populations, ensure that your explanation disaggregates the response between these key population groups).

TWO PAGES MAXIMUM

Gender inequalities exist in all societies and gender inequality related health problems exist for all diseases including tuberculosis. Gender sensitivity starts from the principle of equality of women and men, girls and boys, and addressing specific service needs of each of the groups. Under the existing socio-economic situation in Tanzania, women are more vulnerable to health threats than men. Women in

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general are responsible for the care of children, the sick and the elderly subjecting them to a higher risk of getting infectious disease including tuberculosis and HIV/AIDS.

Social inequalities in access to quality health care also exist world-wide. Marginalized populations tend to get poor quality of health services and most times cannot afford the required treatments. Tanzania has always adhered to the principle of non-discrimination, equality and participation and this is demonstrated in health care delivery services.

The health sector addresses specific gender related health problems. It addresses the girl-child's health, women's health problems and stimulates the involvement of men in family health issues (MDG 4, 5, and 6).

The Global Fund recognizes that men and women have unequal access to health care services. In this proposal health and social welfare equitable service provision will have priority, giving preference to those in the society who are most vulnerable, marginalized groups and those who have the least possibilities to fend for themselves.

In order to enhance gender and social equality and address barriers to accessibility to quality health services in the country, this proposal will focus on the following gender and social sensitive interventions:

- Epidemiological data and outcome indicators will be disaggregated by sex and age so as to understand the disease pattern and how to respond to it. This will highlight social and gender issues to be used in re-planning, monitoring and evaluation of program activities.
- TB/HIV services will be scaled-up beyond the district hospitals to cover certified health facilities in rural settings, prisons, refugee camps and urban settings with a high density of population. All TB and TB/HIV services in the country are offered free of charge enabling all marginalized groups easy access to care and treatment. Pediatric TB/HIV is included as a special package in the TB/HIV training guidelines. Under the current NTLP guidelines, TB screening is provided to those under five years of age who have a sputum smear positive household contact whereby TB treatment or Isoniazid preventive therapy is provided. School age TB and TB/HIV patients are encouraged to attend their relevant clinics at convenient time outside school hours including Saturdays. The NTLP will collaborate with school health programs to train teachers and students on signs and symptoms of TB for early diagnosis and treatment. Youth clinics have been opened in different areas to cater for the needs of teenagers and school-going children in a confidential manner. These free youth-friendly clinics offer education on sexual health, infectious diseases control, HIV counseling and screening, treatment, advice and referral for disease diagnosis and treatment. The NTLP works closely with PMTCT centers and use them as an entry point for TB screening and diagnosis where patients are referred to TB clinics for DOT services.
- Community based DOT is a strategy especially for settings with less access to facility-based DOT. This strategy will also assist severely weak and poor patients to access anti-TB drugs without having to incur the related financial and social obstacles involved with accessing a health facility-based DOT. This proposal will consider providing incentives to those who are poor, nutritionally compromised patients and those living far away from health facilities taking into account accessibility difficulties.
- IEC materials that are gender sensitive and that will highlight health problems that are incurred by women, girls and children will be developed. Women organizations and religious institutions will be empowered to convey key messages on TB and TB/HIV control to the community.
- In order to address the low TB case detection in the country, new diagnostic equipments in the laboratory will be one of the NTLP strategies to increase TB diagnosis in HIV individuals and the pediatric population.

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4.5.5 Strategy to mitigate initial unintended consequences

If this proposal (in s.4.5.1.) includes activities that provide a disease-specific response to health system weaknesses that have an impact on outcomes for the disease, explain:

- the factors considered when deciding to proceed with the request on a disease specific basis; and
- the country's proposed strategy for mitigating any potentially disruptive consequences from a disease-specific approach.

This proposal intends to introduce and scale up the Practical Approach to Lung Health (PAL) during the five years. The proposal also addresses the issue of infrastructure development for scaling up of TB/HIV activities in the country.

Consequences that may result from introduction of the PAL approach is the increased sensitivity in suspecting TB and hence an increase in TB diagnostic requests that include chest x-rays and sputum examinations. This might result in over utilization of the radiology units in different centres and hence a need to increase human resource and supplies required for this activity so as to be able to maintain investigations for other patients requiring radiology services. The intention is to recruit more personnel trained in radiology and more funding to cover the increase in supplies needed to run the radiology units. TB laboratories will be strengthened as requested in this proposal to cater for the increased sputum examinations anticipated.

The request to renovate clinics to accommodate TB/HIV activities and renovate wards to have isolation rooms may create a space problem in the respective health facilities that may be detrimental in the care of other patients. This consequence can be mitigated through relevant negotiations with hospital/clinic management teams on the positive impact of prompt diagnosis and management of TB and HIV.

The scale-up of PAL services in primary health care facilities nationwide (and introduction of more TB/HIV clinics especially in congregate settings) may lead to an inability to conduct quality and timely supportive supervision since there will be need for more human resource and more time for the supervisory visits. This consequence will be addressed by capacity building of the RHMTs and CHMTs to conduct supervision at a timely manner and the staff hiring plan as stipulated in HSS section.

The recruitment of staff to work as TB/HIV district officers (25 districts) and health staff (132 Councils) may lead to human resource moving from working in areas involved with other diseases. This will be resolved by recruiting other personnel from the pool of health workers who are unemployed in the country and those completing medical trainings.

4.6. Links to other interventions and programs

4.6.1. Other Global Fund grant(s)

Describe any link between the focus of this proposal and the activities under any existing Global Fund grant. (*e.g., this proposal requests support for a scale up of ARV treatment and an existing grant provides support for service delivery initiatives to ensure that the treatment can be delivered*).

Proposals should clearly explain if this proposal requests support for the same interventions that are already planned under an existing grant or approved Round 7 or Round 8 proposal, and how there is no duplication. Also, it is important to comment on the reason for implementation delays in existing Global Fund grants, and what is being done to resolve these issues so that they do not also affect implementation of this proposal.

BETWEEN 2 to 4 PAGES

The GFR 9 proposal requests to scale-up, sustain, introduce of new interventions and strengthen services initiated with supports from Global Fund grants Round 3 & 6, and CDC/PEPFAR in implementation of TB and TB/HIV control activities countrywide. It will also build on developments achieved in scaling up ART services in the country. The key linkages between the current grant proposal and the previous ones are elaborated by describing the SDA's and districts/regions covered by each of the available grants.

ROUND 9 – Tuberculosis

GFATM Round 3: This grant supports collaborative TB/HIV activities in 45 districts focusing on expanding access to PITC at the district and community and links to expand TB/HIV services to cover all districts in the country. The grant will expire in 2009. The GFR 3 has inadequate funding to support planned activities in 45 districts such as training and other important activities which were not planned such as expansion of the TB/HIV services within the district. These include infrastructure improvement of DOTS diagnostic and treatment centres to accommodate provision of ART at the district hospitals and improvement of health facilities beyond the district hospital to provide quality TB/HIV services and TB infection control. Other activities which need to be supported include: recruitment and deployment of district TB/HIV officers, quarterly meetings of coordination committees, quarterly technical meetings between health facilities/partners at national and district level, community sensitization and human resource capacity building, and quarterly administrative costs for district TB/HIV officers. Expansion of TB/HIV activities and ensuring sustainability is a valuable initiative given that TB is the leading killer disease among AIDS patients. It is envisaged that the GFR 9 grant will fill in the gaps, complement implementation of TB/HIV activities and ensure sustainability of the services when the grant expires.

The activities planned under the grant suffered from several delays in disbursement of funds to all the implementing agencies, in large part due to weak coordination among implementing agencies and insufficient funding for MoHSW to execute its core tasks and supervisory role. These impediments have been resolved with the introduction of coordination meetings, operationalization of GF has been well understood, the presence of focal people in the programme and there is a flexibility of revision of the plan of operations depending on the need and situation. Therefore the ongoing and the coming GFATM grants are being and will be implemented through this in-built experience.

GFATM Round 6

The GF approved a total of USD 37,118,161; The overall focus of Round 6 is to support the NTLP in the scale up of TB detection including treatment activities and scale up of collaborative TB/HIV activities in 31 districts in seven regions. Under Objective 1, (SDA 1) -support is provided for prevention of HIV in TB patients and (SDA 2) for the provision of ART for TB patients during TB treatment in TB clinic. Under objective 2; (SDA 4) support is provided for community TB care and Behaviour Communication Change (BCC) – Community outreach and (SDA 5) BCC – Media, Under objective 3; (SDA 6),- support public-private Mix, under objective 4; (SDA 7) -support is for control of MDR –TB. The proposed grant (GFR 9) will sustain, scale-up and strengthen services initiated with support from GFR 6, CDC/PEPFAR and introduction of new interventions at all levels. The overall focus is on expansion of DOTS, collaborative TB/HIV services and MDR-TB in 133 district councils and engaging on few Operational researches. The GFR 9 funds will also be used to establish one satellite MDR – TB management site in Dar es Salaam which will accelerate the number of TB patients enrolled for second line drugs thereby reducing further transmission of MDR-TB to the community. PAL will be introduced at all levels of the health system in a phased manner. Activities which are similar and supported by existing grants are not budgeted in this proposal request.

The principal recipient will be the Ministry of Finance and Economic Affairs and the MoHSW together with other implementers will function as the sub-recipients. Funding will further be channeled to sub-sub recipients at the regional and district implementing levels.

4.6.2. Links to non-Global Fund sourced support

Describe any link between this proposal and the activities that are supported through non-Global Fund sources (*summarizing the main achievements planned from that funding over the same term as this proposal*).

Proposals should clearly explain if this proposal requests support for interventions that are new and/or complement existing interventions already planned through other funding sources.

ROUND 9 – Tuberculosis

ONE PAGE MAXIMUM

The NTLP has also been receiving support from CDC/PEPFAR and development partners: German Leprosy and Tuberculosis Relief Agency (GLRA), KNCV (Royal Netherlands TB Foundation) and World Health Organization (WHO). This proposal/grant will also complement existing interventions supported by above mentioned funding and scale up improvement of new interventions.

CDC/PEPFAR support; Under this grant CDC/PEPFAR has allocated funds amounting USD 6,000,000 to support collaborative TB/HIV activities in 57 districts in 10 regions for five years. The grant will end in 2010 with the possibility of renewing on competitive bidding to ensure sustainability. Apart from financial support, CDC also provides technical support and has already deployed 45 TB/HIV district officers. The GFR 9 will complement the initiated and supported activities which have insufficient funds such as: recruitment and deployment of 12 more district TB/HIV officers and transport support for these officers. GFR 9 will provide support for introduction of new interventions; scale up of provision of ART in DOTS diagnostic and treatment centres beyond the districts hospital clinic, infrastructure improvement to accommodate PITC and support for TB control activities under the above mentioned service delivery areas.

Programme for Appropriate Technology in Health (PATH)/USAID: is supporting NTLP to implement collaborative TB/HIV activities in 26 districts and strengthening lab services in Tanzania mainland. Likewise the GFR 9 will fill gaps in infrastructure development, human resource capacity building including strengthening and scaling up of TB control activities.

Achievement under CDC/PEPFAR and PATH/USAID support; to date TB/HIV services have been scaled up in 83 districts, at least 4-5 health facilities in each district are providing TB/HIV services. The policy guideline for collaborative TB/HIV activities and training package was developed. Coordinating committees at regional and districts levels are established and functioning. About 20 DOTS diagnostic and treatment centres at the district level have been upgraded, renovated and refurbished to provide ART. Among them 10 TB clinics have started provision of ART.

Collaborative partners: The NTLP is funded by the Government of Tanzania and other collaborative partners like GLRA, KNCV and WHO provide technical/financial support thereby complementing the efforts layered by the other grants. Novartis Foundation for Sustainable Development (NFSD) through GDF and WHO has given a five year grant of 4-FDC anti TB drugs which will expire in June 2009, with a possibility of extending for two more years. WHO provides mostly technical and financial support in TB/HIV and community TB care. The GLRA supports the NTLP with transport support and supervision costs in the country and complements some TB/HIV activities such as training especially in districts under GFR3 support. GLRA funding will, however, diminish in the coming years due to a shortfall in income from donations in Germany.

4.6.3. Partnerships with the private sector

- (a) The private sector may be co-investing in the activities in this proposal, or participating in a way that contributes to outcomes (even if not a specific activity), if so, summarize the main contributions anticipated over the proposal term, and how these contributions are important to the achievement of the planned outcomes and outputs.

(Refer to the Round 9 Guidelines for a definition of Private Sector and some examples of the types of financial and non-financial contributions from the Private Sector in the framework of a co-investment partnership.)

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More than 60% of Tanzanians in the urban areas seeks healthcare services from private healthcare providers. In general, the private health care sector caters for over 40% of primary health care services in the country (for profit and not for profit-FBOs), while the private for profit alone caters for over 25%. More than 50% of all Tanzania have the private healthcare provider as their first entry point in receiving healthcare whenever they fall sick. (IFC/World Bank Report-2007)

The TB control and TB/HIV activities are conducted within the framework of health system mainly coordinated and driven by the public sector. One component of Health Sector Strategic Plan III 2009- 2013 (HSSP III) is the strengthening of public-private partnerships for the delivery of the essential health package (EHP). In this perspective, the NTLP has started to involve selected facilities of the private voluntary and private for-profit sectors in the provision of TB services; however there is limited involvement of the private sector in the implementation of TB control activities countrywide.

Most of the public-private mix initiatives are in big cities, e.g. Dar es Salaam, Mwanza, Arusha and Mbeya. In GFATM round 6 grants the MoHSW in collaboration with APHFTA and local councils have a set of activities to strengthen public-private partnership and extending collaborative TB/HIV services into the private sector in 31 districts. In this proposal the private sector will be involved in the implementation of most of services delivery areas in scaling up of TB and TB/HIV activities with the emphasis on International Standards of TB Care (ISTC). The contribution provided by the private providers ranges from administrative costs, infrastructures, equipment and human resource. This grant request is intended to support them to be able to provide high quality DOTS and TB/HIV services.

The support through this application will include infrastructure improvement, provision of lab supplies, drugs, microscopes, reagents and training to improve knowledge and skill of their staff. Similarly, monitoring and supervision of TB services in the private sector will be done by NTLP in close collaboration with Association of Private Health Facilities of Tanzania (APHFTA) and the corresponding local council authorities. The basic principles on TB diagnosis and treatment will be determined by ISTC in line with programme treatment guidelines. The DOTS and collaborative TB/HIV services will be provided free of charge as in the public facilities.

The successful implementation of this programme will result into increased case detection, maintaining desirable good treatment outcomes and widening the range of choices of treatment sites among TB care seekers. The monetary estimation of the private sector contribution to TB control and collaborative TB/HIV services in the country has been provided below in respect of the average cost of a patient per attendance day. These estimates are not directly requested in this proposal, and therefore have not been deducted from the overall proposal request detailed in Section 5.

- (b) Identify in the table below the annual amount of the anticipated contribution from this private sector partnership. *(For non-financial contributions, please attempt to provide a monetary value if possible, and at a minimum, a description of that contribution.)*

Population relevant to Private Sector co-investment
(All or part, and which part, of proposal's targeted population group(s)?) →

Both urban and rural population

Contribution Value (in USD or EURO)

Refer to the [Round 9 Guidelines](#) for examples

Organization Name	Contribution Description (in words)	Year 1	Year 2	Year 3	Year 4	Year 5	Total
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ROUND 9 – Tuberculosis

APHTA	Private Providers are estimated to serve average cohort of 11,500 TB patients per year at an extra cost of USD 0.6 per day visit with a total of 19 visits throughout treatment period*. Increment at a rate of 1.2% per year is considered.	98,571	118,286	141,943	170,331	204,398	733,529
<i>[use "Tab" key to add extra rows if needed]</i>							

* Data from PATH, private health services assessment report 2007

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4.7. Program Sustainability

4.7.1. Strengthening capacity and processes to achieve improved tuberculosis outcomes

The Global Fund recognizes that the relative capacity of government and non-government sector organizations (including community-based organizations), can be a significant constraint on the ability to reach and provide services to people (e.g., home-based care, outreach contact, orphan care, etc.).

Describe how this proposal contributes to overall strengthening and/or further development of public, private and community institutions and systems to ensure improved tuberculosis service delivery and outcomes. → [Refer to country evaluation reviews, if available.](#)

ONE PAGE MAXIMUM

The government of Tanzania through the Ministry of Health and Social Welfare recognizes the need to deliver quality and equitable services to its citizens including females, males, girls and boys, with a focus on marginalized populations. Although the government has decentralized primary health care services for 90% of the population to a 10 km walking distance, the community is not yet fully utilizing the available services due to a number of reasons which include: inadequate number of qualified and trained work force, lack of community awareness and stigma among the community to TB patients and little involvement of private health care providers including private health facilities and traditional healers in the management of TB patients. Through historic collaboration with the faith based organizations (FBOs) the government has been able to deliver TB services to hard to reach areas. The experience from Global Fund round six on involvement of private for profit health facilities and community based TB care in 36 districts has significantly increased case notification and treatment success rate.

Another experience from the Social Marketing approach in TB control sponsored by Novartis Foundation for Sustainable Development and piloted in 3 districts namely Arusha, Kahama and Mufindi has shown an increase in community awareness and early health seeking behavior. The unfavorable outcomes in these districts have been halved compared to the baseline data (from 34.2% – 17%).

Basing on these experiences, the proposal is braced to implement the following strategies towards strengthening the capacity of the programme in TB control at all levels:

- Create community awareness and training of traditional healers across the country in the remaining 97 districts
- Development of an ACSM strategy
- Formation of Ex-TB patients clubs to support treatment and intensified case finding
- Identification of celebrities for advocacy in TB control
- Promote patient centered approach across the country
- Involvement of health care providers from private for profit health facilities in line with International Standards in TB Care.

Through the support of this proposal, it is envisaged that community awareness on TB disease will be raised and health seeking behavior will be changed positively resulting into early case detection, adherence to treatment and less stigma and discrimination of TB patients. The participation of communities in TB control through treatment supporters and patient centred approach will reduce the costs incurred by patients to go for daily health facility DOT. The involvement of private for profit health facilities in TB control activities will facilitate managing patients who otherwise would never visit overcrowded public health facilities.

Supporting the community and private health sector through this proposal will ensure skills and capacity for future continuation of programme activities through routine service delivery. Inclusion of programme interventions in Council Health Plans as well as making community structures work, shall enhance local participation and contribute to sustainability. Through this collaborative approach the overall outcome of implementation of this proposal is expected to increase the visibility of the TB program resulting into increased case detection and treatment success hence reduced TB morbidity and mortality.

4.7.2. Alignment with broader developmental frameworks

Describe how this proposal's strategy integrates within broader developmental frameworks such as Poverty Reduction Strategies, the Highly-Indebted Poor Country (HIPC) initiative, the Millennium Development Goals, an existing national health sector development plan, and other important initiatives,

ROUND 9 – Tuberculosis

such as the 'Global Plan to Stop Tuberculosis 2006-2015' for HIV/TB collaborative activities.

ONE PAGE MAXIMUM

The Government of the United Republic of Tanzania is committed to continuously support the stop TB strategies in the country in line with the global initiatives in TB control. In addressing the burden of TB Tanzania has integrated her strategies on the following local and global developmental frameworks

MKUKUTA (in Kiswahili) is the National Strategy for Growth and Reduction of Poverty

The strategy was inaugurated in 2005 and its second objective addresses issues that focus on the improvement of quality of life and social well being of Tanzanians. Economic growth and poverty reduction in this strategy will predictably reduce TB morbidity and mortality as the community will have accessible and equitable TB care, good housing and improved Reproductive and Child Health services.

Tanzania Development Vision 2025

The vision strives to ensure Tanzanians attain a high quality livelihood through universal primary and secondary education, accessible and equitable primary health care, reduction of infant and maternal mortality rates that in totality aim to increase life expectancy.

National Health Policy (2007)

This policy focuses on providing basic services which are equitable, affordable, sustainable, gender sensitive and accessible to all. Addressing these issues will result into a healthy and productive Tanzanian community and ultimately reduce the burden of TB in the country. This policy translates the vision of 2025 and therefore sets a national agenda that needs collective efforts and support of different stakeholders including GFR9.

Primary Health Services Development Programme (2007- 2017)

This strategy is envisioned to be implemented for 10 years and intends to increase the number of qualified midlevel health care providers who work in hard to reach and remote areas. In this programme every village will have a dispensary and a health centre to every ward. In view of that, 3,108 additional dispensaries, 2,074 health centres and 19 district hospitals are expected to be established. Availability and proper management of this health workforce will have an important impact in reducing TB burden. The support requested in this proposal aims at strengthening health systems which will positively contribute to the attainment of this programme.

Health Sector Strategic plan III (2009- 2015)

Tuberculosis is among the most important communicable disease in Tanzania and is one of the targeted diseases for GF support. In this plan the government through the Ministry of Health and Social Welfare is committed to support NTLP strategic and work plans on the ground and those to be developed. Among many others, the priority areas to be supported in this strategic plan include training of health workers in new TB interventions such as DOT expansion, expansion of TB/HIV service, MDR-TB management, procurement of supplies and medicines, introduction of new equipment for diagnosis, and community involvement in TB care and prevention.

The Global Plan to Stop TB 2006 -2015

This plan is addressing the six components of enhancing quality DOTS services, addressing TB/HIV and MDR TB plus other challenges, strengthening health systems, engaging all health care providers in TB control, empowering TB patients and community on TB control and finally operational research. This proposal has been developed in line with the global plan to Stop TB.

Millennium Development Goals (MDGs)

The initiatives that Tanzania has taken are geared towards achieving the 6th MDG which among many others intends to half TB cases relative to 1990 targets. This proposal seeks to scale up best practices learnt from other districts and support the community in increasing case notification and treatment success as a strategy to achieve MDGs by 2015.

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4.8. Measuring impact

4.8.1. Impact Measurement Systems

Describe the strengths and weaknesses of in-country systems used to track or monitor achievements towards national tuberculosis outcomes and measuring impact.

Where one exists, refer to a recent national or external evaluation of the IMS in your description.

ONE PAGE MAXIMUM

In Tanzania, routine M & E systems for health under the MoHSW are HMIS and disease surveillance which can provide overall information of health status of the population. Several key TB programmatic indicators are also captured in this system.

Strengths:

- Availability of an up-to-date National M & E Operational Plan (MH-MES 2006) that is linked to the National Multi-Sectoral Strategic Framework for HIV/AIDS.
- NTLP uses standardized data collection tools at health facility level that capture needed information and standardized reporting tools at district level used for compilation and reporting.
- At the district level data are aggregated quarterly and sent to National level through regions, where reports are entered into an electronic database (MS Access) and analyzed to generate TB programme reports which are then disseminated systematically and transparently.
- From district up to National level, the Electronic TB Register (ETR.Net), a TB and TB/HIV surveillance software which provides easy data collection, data management and analysis, has been installed and training has been conducted to all Regional and Districts TB and leprosy coordinators. Recently deployed TB/HIV officers have also been trained on the use of ETR.net for supporting the DTLCs in data compilation at the district. This electronic based system from district level to national level will improve data quality, management and reduce delays in generating reports and monitoring of patients treatment. ETR.net is under the guidance and leadership of the Data Management Unit at the national level which is well organized and is currently being strengthened through GFR6
- Some supportive supervision visits have been conducted at all levels.
- In addition to the supervision, regional quarterly meetings with DTLCs, TB/HIV officer and host RTLC are conducted to review programme implementation including recording and reporting at district level. Likewise biannual RTLCs meetings are conducted nationally to review overall programme implementation of activities for the past half year.

Weaknesses:

- Records at health facilities are sometimes not filled properly because of inadequate data collection skills triggered by frequent rotation of DOT staff within the health facilities.
- Inadequate mentoring and supportive supervision on data collection, analysis, use and reporting at the sub-national levels and a dearth of systematic feedback to and from the national level downwards.
- At district and regional level there is poor knowledge and skills in interpretation of indicators resulting in weak utilization of data for local planning and monitoring of activities.
- Lack of a feedback mechanisms of data from regions and districts to collecting facilities
- Delays in submission of reports from districts and regions to the national level mainly due to time spent in compiling reports manually in areas where ETR.Net is not fully functional.
- Weak access to computer facilities and low basic computer skills by DTLCs and TB/HIV officers, compounded by low or lack of personnel to maintain and service computers at districts level is a challenge to a fully operational ETR.Net
- Inadequate operations research at all levels

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4.8.2. Avoiding parallel reporting

To what extent do the monitoring and evaluation ('M&E') arrangements in this proposal (*at the PR, Sub-Recipient, and community implementation levels*) use existing reporting frameworks and systems (including reporting channels and cycles, and/or indicator selection)?

HALF PAGE MAXIMUM

This proposal will be aligned with the National Strategy and National M&E Plan. The existing TB specific M&E component under the data management unit at the national level will be used in this proposal. Current human resource from data collection, processing and analysis will be provided with refresher training in data management at all levels. Quarterly DTLC and Biannual RTLC meetings will be used for monitoring and harmonization of data at these levels and they will be supported by this proposal

Supervision from national level to regions is done at least once a year; regions supervise districts on a quarterly basis where by districts supervise health facilities at least once per month. During supervision issues around recording and reporting are usually addressed.

4.8.3. Strengthening monitoring and evaluation systems

What improvements to the M&E systems in the country (including those of the Principal Recipients and Sub-Recipients) are included in this proposal to overcome gaps and/or strengthen reporting into the national impact measurement systems framework?

→ *The Global Fund recommends that 5% to 10% of a proposal's total budget is allocated to M&E activities, in order to strengthen existing M&E systems.*

ONE PAGE MAXIMUM

- This grant will revise and update all data collection tools to include new initiatives which will be incorporated with Global Fund support. Computers will be procured to ensure all districts are fully operational with ETR.net throughout the whole country.
- In view of their high turnover, training and retraining of DOT nurses on filling the updated data records and reporting tools at services delivery points will be conducted. Coordinators (RTLCs, DTLCs) and TB/HIV officers will be given refresher training on the updated ETR.Net so as to facilitate rapid transfer of quality assured data from the districts and regions to the national level.
- Internet connection of Regional and District TB and leprosy coordinators computers will hasten data transfer and significantly reduce report delays. Supportive supervision from the central, regional and district levels and coordinators (DTLCs, RTLC) quarterly and biannual meetings will continue to be the cornerstone of the M&E plan and will be firmly supported by this grant.
- Operational research methodology training to TB coordinators at district, regional and national levels will be provided so as to strengthen operational research at all levels. Better capacity for operations research will encourage feedback and data utilization for planning and service delivery improvement at district level

4.9. Implementation capacity

4.9.1 Principal Recipient(s)

Describe the respective technical, managerial and financial capacities of each Principal Recipient to manage and oversee implementation of the program (or their proportion, as relevant).

In the description, discuss any anticipated barriers to strong performance, referring to any pre-existing assessments of the Principal Recipient(s) other than 'Global Fund Grant Performance Reports'. Plans to address capacity needs should be described in s.4.9.6 below, and included (as relevant) in the work plan and budget.

PR 1	[Ministry of Finance and Economic Affairs Tanzania]
Address	[street address]

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The Ministry of Finance and Economic affairs is by the laws of the land the principal recipient for funds going to sub-recipients in the public sector. On behalf of the Ministry of Finance and Economic affairs, PMU and the ministry of health and social welfare will oversee the implementation of this programme. Tanzania government policy dictates that funds from bi- and multi-lateral donors be channelled through the Ministry of Finance and Economic Affairs (MoFEA) in order to ensure that the funds are recorded and tracked within the national budget and public expenditure reviews. The MoFEA is therefore the only entity in Tanzania with the legal authority to receive funds on behalf of the public sector. Non- government implementers will access funds through the Ministry of Finance and Economic affairs, corresponding leading NGOs and other sub-recipients in their localities. The country's own assessment is that the PR's performance is satisfactory but there are ongoing efforts to improve performance.

Operationally, the MoFEA has nominated a senior officer to deal with Global Fund grants. This senior officer serves as the focal point for all public sector sub-recipients and is responsible for tracking disbursement of the funds. In addition, the MoFEA provides important support to its sub-recipients, including, but not limited, to:

1. Providing support to the LSR and SRs in obtaining tax and duty exemptions from the Tanzania Revenue Authority.
2. Covering any exchange rate losses from foreign exchange fluctuations e.g., in execution of the Malaria Round 4 grant, MoFEA covered a USD \$1.4 Million loss experienced by a sub-grantee.
3. Supporting the auditing costs from the Government Auditor General.

There will be specific personnel that are to be used for the Global Fund rounds to specifically address the bottlenecks in disbursement of funds which have occurred in the past due to lack of accurate timely reporting.

PR 2	Not Applicable
Address	[street address]
[Description]	

PR 3	Not Applicable
Address	[street address]
[Description]	

→ Copy and paste tables above if more than three Principal Recipients

4.9.2 Sub-Recipients	
(a) Will sub-recipients be involved in program implementation?	☒ Yes
	☐ No
(b) If no, why not?	
(c) If yes, how many sub-recipients will be involved?	☒ 1 – 6
	☐ 7 – 20

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	☐ 21 – 50
	☐ more than 50
(d) Are the sub-recipients already identified? <i>(If yes, attach a list of sub-recipients, including details of the 'sector' they represent, and the primary area(s) of their work over the proposal term.)</i>	☒ Yes <i>[Insert Annex Number for list]</i>
	☐ No Answer s.4.9.4. to explain
(e) If yes , comment on the relative proportion of work to be undertaken by the various sub-recipients. If the private sector and/or civil society are not involved, or substantially involved, in program delivery at the sub-recipient level, please explain why.	
MAXIMUM TWO PAGES 1. Ministry of Health and Social Welfare (MoHSW) was the PR in the Global Fund Round 1 Malaria grant, and subsequently served as a SR in later Global Fund rounds. The MoHSW is best placed to be a sub-recipient for implementation of activities in the public sector. It is the technical ministry for health-related TB control activities. It is responsible for the planning and implementation of collaborative TB/HIV services, DOTS, MDR-TB, home-based care programmes and coordination of private health providers. It has a well-developed infrastructure, experience and capacity for technical and financial management for donor funded programmes. There are challenges to the MoHSW in staffing and training of required health workers to carry out quality health services provision for effective and quality TB control services including MDR-TB. The Ministry of Health and Social Welfare will be sub-recipient with the following corresponding sub-sub recipients:- NTLP – Closely collaborate with PMU to provide the technical oversight of the implementation of this programme. Coordinate the organization of all activities concerned with planning, formulation of guidelines and operational manuals, review and updates of data collection and reporting tools. NTLP will also be on the driving seat to undertake monitoring and supervisory activities. MSD – Medical Stores Department is an autonomous department of the Ministry of Health of Tanzania responsible for the sufficient supply of drugs, equipment, medical supplies and health products in the country in both public and faith based organization facilities. In principle, MSD are not allowed to supply medical commodities to private for profit facilities. This agency will deal with all programme matters in regard to procurement, clearance, storage and distribution of anti-TB drugs, lab reagents, lab supplies, equipment and other related health products as indicated in the proposal. Medicals Stores department is already being supported by previous global fund rounds, to strengthen its infrastructure and human resource capacity. Kibong'oto TB Hospital, - This is the designated hospital for managing TB patients with special attention on MDR-TB. The hospital will deal with all in-patient matters for MDR-TB patients as stipulated in the proposal. The rest of the country and central TB reference laboratory will be involved in MDR-TB management at different stages. Academia, research institutions and consultant hospitals – These institutions (both public and private) will be involved in the development of training materials and implementation of the suggested operational researches. 2. Prime Minister's Office Regional Administration and Local Government (PMORALG) has experience in acting as a SR in the previous Global Fund rounds 3. PMORALG serves as the channel for funding to RHMTs and CHMTs for training of HCWs, supportive supervision to lower level facilities, and creating a mentoring network. PMORALG faced some notable implementation challenges until a Global Fund desk was established and staffed with a Senior Officers. The Global Fund desk has since improved implementation performance, reduced fund disbursement times, and generated multiple relevant reports.	

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Support is requested in this proposal to facilitate quarterly meetings with the implementing District and Regional Authorities. The following regional and district entities will function as sub-sub recipients.

RHMTs – The region health management teams are the technical arm of the regional secretariat in 21 regions and function as a link between the ministry of health and social welfare and implementing units, the districts. These teams will:-

- Conduct and supervise training at district level
- Collaborate with zonal training centres to organize and integrate all planned trainings
- Collaborate with the regional secretariat to monitor the implementation process at the district level with all other implementers
- Coordinate and monitor the implementation of the programme in prisons
- Ensure all the reports are prepared and submitted to PMU on time

CHMTs – The council health management teams in 133 district councils function as a management unit of health services at district level. They are responsible for planning and implementation of all health related interventions in collaboration with all available stakeholders. They oversee the management of the district health care delivery system and the services provided in both public and private sectors. During the implementation of this programme, these teams will:-

- Collaborate with all other implementers, including CSOs and NGOs to ensure the successful implementation of this programme
- Closely collaborate with the local NGOs to translate all suggested interventions into action and reaching targeted communities
- Execute all training of health care providers, improvement of infrastructure, scale up of TB/HIV services, strengthening of community empowerment and public-private partnership
- Collect data, prepare reports, analyze them and maximize its utilization in their setting while ensuring the timely submission of quality reports to PMU
- Collaborate with the council management team to monitor and supervise the implementation of this programme at district level

4.9.3. Pre-identified sub-recipients

Describe the past **implementation experience** of key sub-recipients. Also identify any challenges for sub-recipients that could affect performance, and what is planned to mitigate these challenges.

The involvement of public sub-recipients especially RHMTs and CHMTs during GF-R6 has proved to be smooth and cost-effective as most operations are carried out at the district level. However, the areas of supervision, retirement of fund and reporting seem to be challenging. The involvement of CSOs, community, regional secretariat and RHMTs may improve the situation.

Strengthening the capacity of stakeholders is essential in creating the institutional backbone to support the scale up of the key TB/HIV and ISTC interventions discussed in Section 4.5.1.

4.9.4. Sub-recipients to be identified

Explain why some or all of the sub-recipients are not already identified. Also explain the transparent, time-bound process that the Principal Recipient(s) will use to select sub-recipients so as not to delay program performance.

Not Applicable

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4.9.5. Coordination between implementers

Describe how coordination will occur between multiple Principal Recipients, and then between the Principal Recipient(s) and key sub-recipients to ensure timely and transparent program performance.

Comment on factors such as:

- **How Principal Recipients will interact where their work is linked** (e.g., a government Principal Recipient is responsible for procurement of pharmaceutical and/or health products, and a non-government Principal Recipient is responsible for service delivery to, for example, hard to reach groups through non-public systems); and
 - **The extent to which partners will support program implementation** (e.g., by providing management or technical assistance in addition to any assistance requested to be funded through this proposal, if relevant).
- The PMU on behalf of PR and TNCM will coordinate and monitor the smooth implementation of the grant among the sub-recipients and sub sub-recipients.
 - The MoFEA will be the principal recipient with the MoHSW and PMORALG as lead sub-recipients. While the MoHSW will be responsible for coordination duties, the PMORALG and other sub sub-recipients will work on implementation at the field and service delivery.
 - Both development partners like GLRA, CDC, PATH, USAID and the technical agencies like WHO, KNCV, IUATLD etc will be fully involved to provide direct technical assistance backstopping and even guiding the process of soliciting intervention specific required technical assistance as highlighted in this grant request.

4.9.6. Strengthening implementation capacity

The Global Fund encourages in-country efforts to strengthen government, non-government and community-based implementation capacity.

If this proposal is requesting funding for management and/ or technical assistance to ensure strong program performance, summarize:

- (a) the assistance that is planned;**
- (b) the process used to identify needs within the various sectors;
- (c) how the assistance will be obtained on competitive, transparent terms; and
- (d) the process that will be used to evaluate the effectiveness of that assistance, and make adjustments to maintain a high standard of support.

**** (e.g., where the applicant has nominated a second Principal Recipient which requires capacity development to fulfill its role; or where community systems strengthening is identified as a "gap" in achieving national targets, and organizational/management assistance is required to support increased service delivery.)**

TWO PAGE MAXIMUM

Technical Assistance requested on MDR-TB, introduction of new diagnostic tools, ACSM and on practical approach of lung health. Some of the challenges in implementation of the TB programme include:-

1. Inadequate mentoring and supportive supervision on data collection, analysis, use and reporting at the sub-national levels and a dearth of systematic feedback to and from the national level downwards.
2. Limited follow-up mechanisms for identifying and tracking attrition among patients with TB, those on treatment and potential MDR-TB.
3. Inadequate operations research on methods of scaling up proven interventions

Operational research will be needed here. Need assessments and stakeholders consensus meetings will

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be key to decision making. Available survey, supervision and review reports will be revisited and feed into this process.

Assistance will be obtained through consultative dialogue and competitive bidding through the existing government procurement system as guided by the procurement act of 2004. Well-defined Terms of Reference will be drafted, allowing assessment of the effectiveness of the assistance.

Expert opinion and globally accepted logical procedures in relation to the national frameworks will be the basis of making decisions and drawing the way forward as to regards to the specific assistance.

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4.10. Management of pharmaceutical and health products

4.10.1. Scope of Round 9 proposal

Does this proposal seek funding for any pharmaceutical and/or health products?	☐ No → Go to s.4B if relevant, or direct to s.5.
	☐ Yes → Continue on to answer s.4.10.2.

4.10.2. Table of roles and responsibilities

Provide as complete details as possible. (e.g., the Ministry of Health may be the organization responsible for the 'Coordination' activity, and their 'role' is Principal Recipient in this proposal). If a function will be outsourced, identify this in the second column and provide the name of the planned outsourced provider.

Activity	Which organizations and/or departments are responsible for this function? (Identify if Ministry of Health, or Department of Disease Control, or Ministry of Finance, or non-governmental partner, or technical partner.)	In this proposal what is the role of the organization responsible for this function? (Identify if Principal Recipient, sub-recipient, Procurement Agent, Storage Agent, Supply Management Agent, etc.)	Does this proposal request funding for additional staff or technical assistance
Procurement policies & systems	PSU	Procurement and policy making	☐ Yes ☐ No
Intellectual property rights	BRELA	Is a government agency collaborating with TFDA for property rights and it has no direct involvement with this proposal	☐ Yes ☐ No
Quality assurance and quality control	Tanzania Food and Drugs Authority	Will be responsible to ensure quality and safety of both first and second lines anti-TB drugs	☐ Yes ☐ No
Management and coordination <i>More details required in s.4.10.3.</i>	PSU	Management and coordination	☐ Yes ☐ No
Product selection	PSU	Product selection	☐ Yes ☐ No
Management Information Systems (MIS)	PSU	Management information systems	☐ Yes ☐ No
Forecasting	PSU	Forecasting	☐ Yes

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			 No
Procurement and planning	MSD	Procurement of anti-TB drugs, equipment, reagents and supplies in collaboration with NTLP	 Yes  No
Storage and inventory management <i>More details required in s.4.10.4</i>	MSD	Storage and inventory of pharmaceutical and health procured above	 Yes  No
Distribution to other stores and end-users <i>More details required in s.4.10.4</i>	MSD	Distribution to zonal MSD offices whereby CHMTs will collect them for distribution to PHC facilities	 Yes  No
Ensuring rational use and patient safety (pharmacovigilance)	Tanzania Food and Drugs Authority	Ensuring rational use of drugs	 Yes  No

4.10.3. Past management experience

What is the past experience of each organization that will manage the process of procuring, storing and overseeing distribution of pharmaceutical and health products?

Organization Name	PR, sub-recipient, or agent?	Total value procured during last financial year <i>(Same currency as on cover of proposal)</i>
MSD	Agent	USD 594,002
<i>[use the "Tab" key to add extra rows if more than four organizations will be involved in the management of this work]</i>		

4.10.4. Alignment with existing systems

Describe the extent to which this proposal uses existing country systems for the management of the additional pharmaceutical and health product activities that are planned, including pharmacovigilance systems. If existing systems are not used, explain why.

ONE PAGE MAXIMUM

The country has a well functioning Regulatory Authority (i.e. Tanzania Food and Drugs Authority) which is responsible for controlling the quality, safety and efficacy of food, drugs, cosmetics and medical devices. The functions of this Authority include:

- Regulating the importation, manufacture, labelling, sale and distribution of food, drugs, cosmetics, herbal drugs and medical devices or any materials or substances used in the manufacture of such products.
- Ensuring that evidence of existing and new adverse events, interactions and information about pharmacovigilance of products being monitored, are analysed and acted upon.
- Ensuring that clinical trials on drugs, medical devices and herbal drugs are being conducted in accordance with prescribed standards.
- Promoting rational use of drugs, medical devices and herbal drugs.

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(e) Prescribing standards of quality.

The pharmacovigilance system of TFDA was established in 1993. It consists of the national centre (i.e. TFDA HQ) and zonal pharmacovigilance centers located at 4 referral hospitals in the country (i.e. Kilimanjaro Christian Medical Centre (KCMC) - Kilimanjaro, Muhimbili National Hospital (MNH) – Dar-es-Salaam, Mbeya Regional Hospital – Mbeya & Bugando Medical Centre (BMC) - Mwanza.

Two approaches of pharmacovigilance exist:

- Spontaneous reporting system
- Active surveillance system

The spontaneous reporting system uses prepaid ADR reporting forms (Yellow forms) developed by TFDA to collect data. Forms exist in English and Kiswahili languages. TFDA distributes forms to zonal centers (managed by regional pharmacists) which then circulate them to healthcare professionals for filling in. Completed forms are being forwarded by the zonal centers to TFDA HQ for causality assessment. Part of causality assessment is also done by healthcare providers at health facilities. Web based reporting also exists through <http://www.tfda.or.tz/reportadr.htm>.

The TFDA has also subscribed to the WHO pharmacovigilance database (i.e. Vigiflow) which has been established to link national centre, zonal centers and other Member States. Data collected in Tanzania is entered into the Vigiflow and sent to WHO as part of global sharing of drug safety information. Data collected from zonal centers is cleaned and validated at the national center before being sent to WHO.

An active surveillance system using cohort event monitoring (CEM) approach is also being implemented by TFDA for some medicines. CEM is a prospective, non-interventional, longitudinal, cohort observation of clinical events associated with a particular medicine. It requires recording of all clinical events irrespective of causality. The CEM will be a good methodology to adopt for monitoring safety of especially second line anti-TB drugs. This will be easy to integrate in the current National Tuberculosis and Leprosy Programme (NTLP) of the Ministry of Health and Social Welfare. Directly observed therapy system (DOTS) implemented by NTLP will also compliment in ensuring effective safety monitoring.

The TFDA also has a post-marketing surveillance (PMS) system which involves random sampling of drug products at ports of entry (PoE), health facilities and drug outlets. Products sampled and collected are usually screened using GPHF Mini Lab Test Kits stationed at various zonal offices. Screening involves visual and physical inspections, simple disintegration test (for solid dosage forms) and Thin Layer Chromatography (TLC). After screening samples are normally sent to TFDA Laboratory located at headquarter offices for confirmatory testing. The system enables screening, identification and detection of poor quality, defective and counterfeit products.

4.10.5. Storage and distribution systems

(a) Which organization(s) have primary responsibility to provide storage and distribution services under this proposal?	☒ National medical stores or equivalent
	☐ Sub-contracted national organization(s) (specify)
	☐ Sub-contracted international organization(s) (specify)
	☐ Other: (specify)

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(b)	For storage partners, what is each organization's current storage capacity for pharmaceutical and health products? If this proposal represents a significant change in the volume of products to be stored, estimate the relative change in percent, and explain what plans are in place to ensure increased capacity.
Not Applicable	
(c)	For distribution partners, what is each organization's current distribution capacity for pharmaceutical and health products? If this proposal represents a significant change in the volume of products to be distributed or the area(s) where distribution will occur, estimate the relative change in percent, and explain what plans are in place to ensure increased capacity.
Not Applicable	

4.10.6. Pharmaceutical and health products for initial two years

Complete 'Attachment B-Tuberculosis' to this Proposal Form, to list all of the pharmaceutical and health products that are requested to be funded through this proposal.

Also include the expected costs per unit, and information on the existing 'Standard Treatment Guidelines' (STGs). **However**, if the pharmaceutical products included in 'Attachment B-Tuberculosis' are not included in the current national, institutional or World Health Organization STGs, or Essential Medicines Lists (EMLs), describe below the STGs that are planned to be utilized, and the rationale for their use.

In addition to WHO-STGs, the following operational manuals will be utilized.

- NTLP manual which include sections on TB standard treatment guidelines
- National Health Laboratory Strategic plan 2009 - 2015
- AFB microscopy manual
- EQA guidelines
- MDR – TB guidelines

4.10.7. Multi-drug-resistant tuberculosis

Is the provision of treatment of multi-drug-resistant tuberculosis included in this tuberculosis proposal?



Yes

In the budget, include USD 50,000 per year over the full proposal term to contribute to the costs of Green Light Committee Secretariat support services.



No

Do not include these costs

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4B. PROGRAM DESCRIPTION – HSS CROSS-CUTTING INTERVENTIONS

Optional section for applicants

SECTION 4B CAN ONLY BE INCLUDED IN ONE DISEASE IN ROUND 9 *and only if*:

- *The applicant has identified gaps and constraints in the health system that have an impact on HIV, tuberculosis and malaria outcomes;*
- *The interventions required to respond to these gaps and constraints are 'cross-cutting' and benefit more than one of the three diseases (and perhaps also benefit other health outcomes); and*
- *Section 4B is not also included in the HIV or malaria proposal*

Read the [Round 9 Guidelines](#) to consider including HSS cross-cutting interventions.

'Section 4B' can be downloaded from the Global Fund's website [here](#) if the applicant intends to apply for 'Health systems strengthening cross-cutting interventions' ('HSS cross-cutting interventions').

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Clarified Section 5.5

5. FUNDING REQUEST

5.1. Financial gap analysis - Tuberculosis

→ Summary Information provided in the table below should be explained further in sections 5.1.1 – 5.1.3 below.

Financial gap analysis (same currency as identified on proposal coversheet)								
Note ➔ Adjust headings (as necessary) in tables from calendar years to financial years (e.g., FY ending 2008 etc.) to align with national planning and fiscal periods								
	Actual		Planned		Estimated			
	2007/2008	2008/2009	2009/2010	2010/2011	2011/2012	2012/2013	2013/2014	2014/2015
Tuberculosis program funding needs to deliver comprehensive diagnosis, treatment and care and support services to target populations								
Line A ➔ Provide annual amounts	11,425,842	18,000,000	36,472,582	43,767,098	43,767,098	43,767,098	52,520,518	52,520,518
Line A.1 ➔ Total need over length of Round 9 Funding Request						(combined total need over Round 9 proposal term)		
Current and future resources to meet financial need								
Domestic source B1: Loans and debt relief (World Bank)	0	0	0	1,338,000	1,338,000	1,334,000	1,330,000	1,330,000
Domestic source B2 National funding resources	950,000	925,000	6,741,667	6,741,667	6,591,667	6,591,667	7,366,667	7,366,667
Domestic source B3 Private Sector contributions (national)				98,571	118,286	141,943	170,331	204,398
Total of Line B entries ➔ Total current & planned DOMESTIC (including debt relief) resources:	950,000	925,000	6,741,667	8,178,238	8,047,953	8,067,610	8,866,998	8,901,065
External source C 1 (CDC/PEPFAR, GLRA, WHO/TDR and others)	5,583,528	6,691,000	6,934,431	6,977,460	7,326,960	4,736,960	5,086,460	5,266,460

ROUND 9 – Tuberculosis

Financial gap analysis <i>(same currency as identified on proposal coversheet)</i>								
Note → Adjust headings (as necessary) in tables from calendar years to financial years (e.g., FY ending 2008 etc.) to align with national planning and fiscal periods								
	Actual		Planned		Estimated			
	2007/2008	2008/2009	2009/2010	2010/2011	2011/2012	2012/2013	2013/2014	2014/2015
External source C2 <i>(provide source name)</i>								
External source C3 Private Sector contributions (International)				0	0	0	0	0
Total of Line C entries → Total current & planned EXTERNAL (non-Global Fund grant) resources:	5,583,528	6,691,000	6,934,431	6,977,460	7,326,960	4,736,960	5,086,460	5,266,460
In line D below, insert additional separate lines for each separate Global Fund grant. This will ensure that you show information on different Global Fund grants.								
Line D: Annual value of all existing Global Fund grants for same disease: Include unsigned 'Phase 2' amounts as "planned" amounts in relevant years	5,192,072	7,740,872	5,796,802	6,035,584	5,889,044	0	0	0
Line E → Total current and planned resources (i.e. Line E = Line B total + Line C total + Line D Total)	7,517,072	10,915,872	19,472,900	21,092,711	21,145,671	12,662,627	13,783,127	13,963,127
Calculation of gap in financial resources and summary of total funding requested in Round 9 <i>(to be supported by detailed budget)</i>								
Line F → Total funding gap (i.e. Line F = Line A – Line E)	3,908,770	7,084,128	16,999,682	23,160,837	22,621,427	31,104,471	38,737,391	38,557,391
Line G = Round 9 tuberculosis funding request <i>(same amount as requested in table 5.3 for this disease)</i>				23,095,100	20,950,085	19,512,614	18,493,361	17,375,642

ROUND 9 – Tuberculosis

Part H – 'Cost Sharing' calculation for **Lower-middle income** and **Upper-middle income** applicants

In Round 9, the total maximum funding request for tuberculosis in Line G is:

- (a) For **Lower-Middle income countries**, an amount that results in the Global Fund's overall contribution (all grants) to the national program reaching not more than 65% of the national disease program funding needs over the proposal term; and*
- (b) For **Upper-Middle income countries**, an amount that results in the Global Fund overall contribution (all grants) to the national program reaching not more than 35% of the national disease program funding needs over the proposal term.*

Line H → Cost Sharing calculation as a percentage (%) of overall funding from Global Fund

Cost sharing =
$$\frac{\text{(Total of Line D entries over 2010-2014 period + Line G Total)}}{\text{Line A.1}} \times 100$$

Line A.1

Not Applicable

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5.1.1. Explanation of financial needs – LINE A in table 5.1

Explain how the annual amounts were:

- developed (e.g., through costed national strategies, a Medium Term Expenditure Framework [MTEF], or other basis); and
- budgeted in a way that ensures that government, non-government and community needs were included to ensure fully implementation of country's tuberculosis program and strategy.

The estimated annual amounts for financial needs have been developed based on the sums of separate plans and budgets by different sources. The STOP TB Global Plan has been used as guidance in arriving to the current separate plans and budgets. These separate plans will be consolidated into the next National Tuberculosis Strategic Plan for five years replacing the one which is ending June 2009. You will notice that the estimates for the requirements/needs have doubled between 2008/09 and 2009/10 this is because of three factors; introduction of the new diagnostic tools, TB case finding approaches and scaling up of collaborative TB/HIV and MDR-TB services. Moreover, the estimates beyond 2011/12 have been calculated by factoring proportional increase anticipating that the volume of services will increase in the same suggested percentages. Each year the government develops MTEF for three year cycles with projected financial needs that are reflected in the priority areas of the National Strategy for Growth and Reduction of Poverty (NSGRP). TB control is among the priority areas in the MTEF health section. On the basis of this and the Universal Access Targeting; the demand for all TB control activities, even those implemented by the different stakeholders, was projected.

5.1.2. Domestic funding – 'LINE B' entries in table 5.1

Explain the processes used in country to:

- prioritize domestic financial contributions to the national tuberculosis program (including HIPC [Heavily Indebted Poor Country] and other debt relief, and grant or loan funds that are contributed through the national budget); and
- ensure that domestic resources are utilized efficiently, transparently and equitably, to help implement treatment, diagnosis, care and support strategy at the national, sub-national and community levels.

Line A represents the financing needs to implement Tanzania's national TB programme as indicated in the current separate TB plans and budgets and as anticipated to be reflected in the forthcoming NTLP Strategic Plan 2009/10 – 2014/15.

Round 9 focuses on strategies built on the basis of Stop TB Global plan 2006 – 2015 with emphasis on universal access to TB care, prevention, and community based support services. All external sources of funding for TB control and collaborative TB/HIV services have been assessed and projected spending documented as shown in section 5.1.

Domestic financing is prioritized on the basis of the burden of the disease and its influence on achievement of the NSGRP, MDGs and WHO TB control targets. The government has developed and implemented an integrated financial management system at the central government level. The implementation of this system has been done by installing an electronic accounting system (EPICOR) and Planning and Reporting Program (Plan REP). EPICOR provides for recording of expenditures based on budgeted items. It also simplifies the generation of financial reports. Plan REP ensures implementation according to the strategic national plans and detects any deviations from the plans.

At PMU level, MOHSW as the LSR in implementing Global Fund projects for TB has procured an accounting software under GFR6 that was installed in September 2008. Effective implementation is on-going with recruitment and training of accounts staff. The software is now undergoing a test run and full operationalization is to start in the near future.

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A number of structures have been put in place to ensure that public resources are utilized efficiently. All Ministry Departments and Agencies (MDAs) have internal audit units while the National Audit Office conduct external audits to most public accounts. Furthermore, there are several Parliamentary committees; namely the Public Accounts Committee, Local Authorities Accounts Committee and the Parastatal Organizations Accounts Committee. These are a combination of oversight and watchdog structures that serve to monitor mismanagement of funds by ensuring transparent and proper utilization of funds by the government and its institutions. EPICOR is capable of and has improved enforcement of compliance to spending according to budgeted amounts.

5.1.3. External funding *excluding Global Fund – 'LINE C' entries in table 5.1*

Explain any changes in contributions anticipated over the proposal term (*and the reason for any identified reductions in external resources over time*). Any current delays in accessing the external funding identified in table 5.1 should be explained (including the reason for the delay, and plans to resolve the issue(s)).

The Global financial crisis is expected to cause significant reduction in external funding in the future. Of recent, a few changes have been realized. GLRA has been giving support through transport and supervision activities while giving technical support. Donations towards the Fund have dwindled and hence GLRA is to substantially reduce its future support to NTLP-Tanzania. Some of the NTLP activities traditionally supported by GLRA have been included in this proposal request for sustaining the good programme performance achieved to date. The caution of expected decline in programme support has also been documented in the recently published NTLP end term (2004-2009) review report by external consultants. CDC/PEPFAR has been providing support in TB/HIV activities, support which is not stable and guaranteed. Moreover, most partners give annual commitments which do not provide long term accurate information to support planning and implementation of programme activities.

5.2. Detailed Budget

Suggested steps in budget completion:

1. **Submit a detailed proposal budget in Microsoft Excel format as a clearly numbered annex.** Wherever possible, use the same numbering for budget line items as the program description.
 - **FOR GUIDANCE ON THE LEVEL OF DETAIL REQUIRED** (*or to use a template if there is no existing in-country detailed budgeting framework*) **refer to the budget information available at the following link:** <http://www.theglobalfund.org/en/rounds/9/single/#budget>
2. Ensure the detailed budget is consistent with the detailed workplan of program activities.
3. From that detailed budget, **prepare a 'Summary by Objective and Service Delivery Area'** (s.5.3.)
4. From the same detailed budget, **prepare a 'Summary by Cost Category'** (s.5.4.)
5. Do not include any CCM or Sub-CCM operating costs in Round 9. This support is now available through a separate application for funding made direct to the Global Fund (and not funded through grant funds). The application is available at: <http://www.theglobalfund.org/en/ccm/>

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5.3. Summary of detailed budget by objective and service delivery area

Objective Number	Service delivery area <i>(Use the same numbering as in program description in s.4.5.1.)</i>	Year 1	Year 2	Year 3	Year 4	Year 5	Total
5	SDA 5.1 ACSM Advocacy Communication and Social Mobilization	2,175,121	2,469,475	924,383	2,893,858	1,000,762	9,463,599
6	SDA 6.1 All care providers	685,123	493,536	523,149	554,537	587,809	2,844,154
5	SDA 5.2 Community TB care	1,106,937	950,902	543,981	576,620	611,217	3,789,657
1	SDA 1.1 High Quality DOTS	2,569,447	1,530,521	5,573,955	5,604,683	5,940,965	21,219,571
1	SDA 1.2 Improving diagnosis	1,143,576	381,107	409,771	434,357	321,496	2,690,307
1	SDA 1.3 M&E	2,348,783	2,206,680	2,486,567	2,479,426	2,739,615	12,261,071
3	SDA 3.1 MDR-TB	2,740,785	2,139,487	2,298,767	1,831,342	1,894,183	10,904,564
7	SDA 7.1 Operation research	17,920	194,563	20,225	-	-	232,708
4	SDA 4.1 PAL (Practical Approach to Lung Health)	1,313,934	1,070,464	1,001,428	1,134,096	1,125,204	5,645,126
2	SDA 2.1 TB/HIV	8,993,474	9,513,350	5,730,388	2,984,442	3,154,391	30,376,045
Round 9 tuberculosis funding request:		23,095,100	20,950,085	19,512,614	18,493,361	17,375,642	99,426,802

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5.4. Summary of detailed budget by cost category *(Summary information in this table should be further explained in sections 5.4.1 – 5.4.3 below.)*

Avoid using the "other" category unless necessary – read the [Round 9 Guidelines](#)

	<i>(same currency as on cover sheet of Proposal Form)</i>					
	Year 1	Year 2	Year 3	Year 4	Year 5	Total
Human resources	340,530	467,952	495,723	525,466	556,994	2,386,665
Technical and Management Assistance	738,570	958,452	829,947	815,159	820,923	4,163,051
Training	5,353,430	2,502,070	533,671	638,274	456,817	9,484,262
Health products and health equipment	1,015,472	39,148	41,497	43,987	46,626	1,186,730
Pharmaceutical products (medicines)	1,022,187	1,030,518	4,757,434	5,042,882	5,345,454	17,198,475
Procurement and supply management costs	316,009	255,591	271,817	288,127	305,415	1,436,959
Infrastructure and other equipment	6,524,167	7,199,738	5,115,494	1,466,737	1,554,741	21,860,877
Communication Materials	1,915,288	3,051,759	1,562,815	3,503,624	1,718,105	11,751,591
Monitoring & Evaluation	1,870,578	1,700,824	1,950,360	1,978,016	2,137,132	9,636,910
Living Support to Clients/Target Populations	1,489,132	1,617,417	1,699,644	1,801,623	1,909,720	8,517,536
Planning and administration	2,008,195	1,594,981	1,690,679	1,792,121	1,890,530	8,976,506
Overheads	381,542	404,435	428,701	454,423	481,688	2,150,789
Other	120,000	127,200	134,832	142,922	151,497	676,451
Round 9 tuberculosis funding request <i>(Should be the same annual totals as table 5.2)</i>	23,095,100	20,950,085	19,512,614	18,493,361	17,375,642	99,426,802

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5.4.1. Overall budget context

Briefly explain any significant variations in cost categories by year, or significant five year totals for those categories.

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There are no significant variations, however there is declining trend. The reason for the decline is that most of the investment costs are undertaken in the initial years of the implementation of this plan. This includes procurement of medical equipment, training of staff and rehabilitation of infrastructure.

5.4.2. Human resources

In cases where 'human resources' represents an important share of the budget, summarize: (i) the basis for the budget calculation over the initial two years; (ii) the method of calculating the anticipated costs over years three to five; and (iii) to what extent human resources spending will strengthen service delivery.

(Useful information to support the assumptions to be set out in the detailed budget includes: a list of the proposed positions that is consistent with assumptions on hours, salary etc included in the detailed budget; and the proportion (in percentage terms) of time that will be allocated to the work under this proposal.

→ *Attach supporting information as a clearly named and numbered annex*

HALF PAGE MAXIMUM

Not Applicable: Human resources account for only 2.4% of this proposal

5.4.3. Other large expenditure items

If other 'cost categories' represent important amounts in the summary in table 5.4, (i) explain the basis for the budget calculation of those amounts. Also explain how this contribution is important to implementation of the national tuberculosis program.

→ *Attach supporting information as a clearly named and numbered annex*

HALF PAGE MAXIMUM

There are five cost categories with large expenditures, these are; pharmaceutical products, infrastructure and medical equipment, communication materials, training and M&E. With regard to pharmaceutical products and medical equipment a price list from medical stores department and estimated quantities of services were used. Cost for infrastructure was established on the standards costing of the bills of quantities provided by building section of the MoHSW. The cost for communication materials are derived out of the prices provided by different mass media, such as proposed air time rates by media owners association of Tanzania (MOAT), rates charged for space in print media and estimates for traditional media. The costing estimates for training sessions were based on the government rates. Moreover, estimates for M&E took care of the specialized type of monitoring within TB programme which could not be undertaken into the integration of general health services. The five components are key/core in the delivery of TB services. Pharmaceuticals and equipment form a major component of diagnosis and treatment. Infrastructure rehabilitation would provide more space, confidentiality to patients and contribute to the TB infection control strategy. Communication component of ACSM will increase early case detection and uptake of TB treatment. Training is necessary as we are introducing new globally accepted approaches (as per Global plan to Stop TB) and diagnostic tools. Monitoring and evaluation will provide the progress of implementation of this plan.

5.5. Funding requests in the context of a common funding mechanism

In this section, **common funding mechanism** refers to situations where all funding is contributed into a common fund for distribution to implementing partners.

Do not complete this section if the country pools, for example, procurement efforts, but all other

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funding is managed separately.

5.5.1. Operational status of common funding mechanism
Briefly summarize the main features of the common funding mechanism, including the fund's name, objectives, governance structure and key partners. → <i>Attach, as clearly named and numbered annexes to your proposal, the memorandum of understanding, joint Monitoring and Evaluation procedures, the latest annual review, accountability procedures, list of key partners, etc.</i>
Not Applicable
5.5.2. Measuring performance
How often is program performance measured by the common funding mechanism? Explain whether program performance influences financial contributions to the common fund.
Not Applicable
5.5.3 Additionality of Global Fund request
Explain how the funding requested in this proposal (<i>if approved</i>) will contribute to the achievement of outputs and outcomes that would not otherwise have been supported by resources currently or planned to be available to the common funding mechanism. <i>If the focus of the common fund is broader than the tuberculosis program, applicants must explain the process by which they will ensure that funds requested will contribute towards achieving impact on tuberculosis outcomes during the proposal term.</i>
Not Applicable

5B. FUNDING REQUEST – HSS CROSS-CUTTING INTERVENTIONS

Applying for funding for HSS cross-cutting interventions is optional in Round 9

SECTION 5B CAN ONLY BE INCLUDED IN ONE DISEASE IN ROUND 9 and only if this disease includes the applicant's programmatic description of HSS cross-cutting interventions in s.4B.

Read the [Round 9 Guidelines](#) to consider including HSS cross-cutting interventions

Download 'Section 5B' from the Global Fund website [here](#) if the applicant intends to apply for 'Health systems strengthening cross-cutting interventions' ('HSS cross-cutting interventions') in Round 9 and has completed section 4B and included that section in the Tuberculosis proposal sections.

Proposal checklist – Section 3 to 5

Tuberculosis

Section 3 and 4: Program Description		List Annex Name and Number
Section 4.1.a	NTLP Strategic Plan 2004-2009 (NSP)	1
Section 4.1.b	Stop TB Global Plan 2006 - 2015	
Sections 4.1.b and 4.4	NTLP end Term 2004-2009 Review Report	2
Section 4.8.1	NTLP Annual Report 2007	3
Section 4.8.1	Global Fund M&E System Strengthening Tool (MESST)	
Section 4.10.4	Health Sector Strategic plan (HSSP III) 2009 -2013	4
Section 4.2.1.b	WHO TB Global report 2008	
Sections 4.2.1.a and 4.4	National Bureau of Statistics (NBS): National Projections Vol X11, (selected Tables) Oct 2006	GF Round 7& 8
Annexes	List of Sub Recipient and Sub Sub-Recipients	5
4.9.1	List of Acronyms and Abbreviations	6
4.9.2	List of sub-recipients already identified (including name, sector they represent, and SDA(s) most relevant to their activities during the proposal term)	
4.10.6	A completed 'List of Pharmaceutical and Health Products' by disease (if applicable).	Attachment B
Section 4B: HSS Cross-cutting (once only in whole country proposal)		List Annex Name and Number
4B.2	A completed separate HSS cross-cutting 'Performance Framework' (or add a separate "worksheet" to the disease 'Performance Framework' under which s. 4B is submitted) Refer to the M&E Toolkit for help in completing this table.	Attachment A

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4B.2	A detailed separate HSS cross-cutting Work Plan (or add a separate “worksheet” to the disease Work Plan under which s. 4B is submitted) (quarterly information for the first two years and annual information for years 3, 4 and 5).	Work plan
Section 5: Financial Information		List Annex Name and Number
5.2	A ‘detailed budget’ (quarterly information for the first two years, and annual information for years 3, 4 and 5)	Detailed Budget
5.4.2	Information on basis for budget calculation and diagram and/or list of planned human resources funded by proposal (only if relevant)	
5.4.3	Information on basis of costing for ‘large cost category’ items	
5.5.1 <i>(if common funding mechanism)</i>	Documentation describing the functioning of the common funding mechanism	
5.5.2 <i>(if common funding mechanism)</i>	Most recent assessment of the performance of the common funding mechanism	
Section 5B: HSS Cross-cutting financial information		List Annex Name and Number
5B.1	A separate HSS cross-cutting ‘detailed budget’ (or add a separate “worksheet” to the disease ‘detailed budget’ under which s. 4B is submitted). Quarterly information for the first two years, and annual information for years 3, 4 and 5).	Detailed Budget
5B.4.2	Information on basis for budget calculation and diagram and/or list of planned human resources funded by proposal (only if relevant)	
5B.4.3	Information on basis of costing for ‘large cost category’ items	
Other documents relevant to sections 3, 4 and 5 attached by Applicant:		List Annex Name and Number

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